



# NATIONAL COMMITMENTS AND POLICY INSTRUMENT 2024

for reporting period January - December 2023

## National Commitments and Policy Instrument: Part A

\* The guidelines for the NCPI define the terms marked with an asterisk (\*).

### 1. Combination prevention for all

- Reduce new HIV infections to under 370 000 by 2025.
- Ensure that 95% of people at risk of HIV infection—within all epidemiologically relevant groups, age groups and geographic settings—have access to and use appropriate, prioritized, person-centred and effective combination prevention options.
- Reduce the number of new HIV infections among adolescent girls and young women to below 50 000 by 2025.
- Ensure availability of pre-exposure prophylaxis (PrEP) for 10 million people at substantial risk of HIV and post-exposure prophylaxis (PEP) for people recently exposed to HIV by 2025.
- 95% of people within humanitarian settings at risk of HIV use appropriate, prioritized, people-centred and effective combination prevention options.

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#### HIV prevention for sex workers

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##### 1. Does your country have a national strategy that includes actions to reduce new infections among sex workers and provide services to sex workers and their clients?

- ☒ Yes  
☐ No

##### 1.1 If yes, does the national strategy include (select all that apply):

- ☒ Community empowerment and capacity-building for sex worker organizations  
☒ Community-based outreach and services for sex workers and their clients  
☒ Distribution of condoms for sex workers and their clients  
☒ Sexually transmitted infection prevention, screening and treatment services  
☐ Psychosocial counselling or mental health services for sex workers and their clients  
☐ Hepatitis prevention, testing and treatment services for sex workers and their clients  
☒ Clinical services for sex workers and their clients  
☒ Legal support services for sex workers  
☒ Actions to address violence, including psychological first aid  
☒ Post-exposure prophylaxis (PEP)  
☒ Actions to reduce stigma and discrimination

##### 2. Are there national standard operating procedures (SOPs) or detailed national implementation guidelines for organizations implementing HIV prevention programmes for sex workers?

- ☒ Yes, national SOPs apply for all organizations  
☐ No, there are SOPs only for organizations under specific programmes  
☐ No, all organizations use their own procedures

##### 3. Has your country set national HIV prevention targets for 2025 for sex workers and their clients?

- ☒ Yes  
☐ No

##### 3.1 If yes, what are the national prevention targets for 2025 for sex workers and their clients?

- 3.1.a Annual number of new HIV infections among sex workers and their clients: n/a  
 3.1.b Percentage of sex workers reached by HIV prevention services: 52%  
 3.1.c Percentage of clients of sex workers reached by HIV prevention services: n/a
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#### HIV prevention for gay men and other men who have sex with men

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4. Does your country have a national strategy that includes actions to reduce new infections among and provide services to gay men and other men who have sex with men?

- ☒ Yes  
☐ No

4.1 If yes, does the national strategy include for gay men and other men who have sex with men (select all that apply):

- ☒ Community empowerment and capacity-building for organizations of gay men and other men who have sex with men  
☒ Community-based outreach and services  
☒ Distribution of condoms and condom-compatible lubricants  
☐ Hepatitis prevention, testing and treatment services  
☒ Clinical services  
☒ Sexually transmitted infection prevention, screening and treatment services  
☒ Psychosocial counselling and/or mental health services  
☒ Legal support services  
☒ Actions to address violence, including psychological first aid  
☒ Post-exposure prophylaxis (PEP)  
☒ Actions to reduce stigma and discrimination

5. Are there national standard operating procedures (SOPs) or detailed national implementation guidelines for organizations implementing HIV prevention programmes for gay men and other men who have sex with men?

- ☒ Yes, national SOPs apply for all organizations  
☐ No, there are SOPs only for organizations under specific programmes  
☐ No, all organizations use their own procedures

6. Has your country set national prevention targets for 2025 for gay men and other men who have sex with men?

- ☒ Yes  
☐ No

6.1 If yes, what are the national prevention targets for 2025 for gay men and other men who have sex with men?

- 6.1.a Annual number of new HIV infections among gay men and other men who have sex with men: n/a  
6.1.b Percentage of gay men and other men who have sex with men reached by prevention programmes: 45%

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#### HIV prevention for transgender people

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7. Does your country have a national strategy that includes actions to reduce new infections among and provide services to transgender people?

- ☒ Yes  
☐ No

7.1 If yes, does the national strategy include for transgender people (select all that apply):

- ☒ Community empowerment and capacity-building for organizations of transgender people  
☒ Community-based outreach and services  
☒ Distribution of condoms and condom-compatible lubricants  
☒ Sexually transmitted infection prevention, screening and treatment services  
☒ Psychosocial counselling and/or mental health services  
☐ Hepatitis prevention, testing and treatment services  
☒ Clinical services  
☒ Legal support services  
☒ Actions to address violence, including psychological first aid  
☒ Post-exposure prophylaxis (PEP)  
☒ Actions to reduce stigma and discrimination  
☐ Gender-affirming health services

8. Are there national standard operating procedures (SOPs) or detailed national implementation guidelines for organizations implementing HIV prevention programmes for transgender people?

- ☒ Yes, national SOPs apply for all organizations  
☐ No, there are SOPs only for organizations under specific programmes  
☐ No, all organizations use their own procedures
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**9.** Has your country set national prevention targets for 2025 for transgender people?

- ☒ Yes  
☐ No

**9.1** If yes, what are the national prevention targets for 2025 for transgender people?

**9.1.a** Annual number of new HIV infections among transgender people: n/a

**9.1.b** Percentage of transgender people reached by prevention programmes: 15%

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#### HIV prevention for people who inject drugs

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**10.** Does your country have a national strategy that includes actions to reduce new infections among and provide services to people who inject drugs?

- ☐ Yes  
☒ No

**10.1** If yes, does the national strategy include for people who inject drugs (select all that apply):

- ☐ Community empowerment and capacity-building for organizations of people who inject drugs  
☐ Community-based outreach and services  
☐ Distribution of condoms and condom-compatible lubricants  
☐ Sexually transmitted infection prevention, screening and treatment services  
☐ Psychosocial counselling and/or mental health services  
☐ Hepatitis prevention, testing and treatment services  
☐ Clinical services  
☐ Legal support services  
☐ Actions to address violence, including psychological first aid  
☐ Post-exposure prophylaxis (PEP)  
☐ Actions to reduce stigma and discrimination  
☐ Needle and syringe programmes  
☐ Opioid agonist maintenance therapy  
☐ Community distribution of naloxone for overdose prevention
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**11.** Are there national standard operating procedures (SOPs) or detailed national implementation guidelines for organizations implementing HIV prevention programmes for people who inject drugs?

- ☐ Yes, national SOPs apply for all organizations  
☐ No, there are SOPs only for organizations under specific programmes  
☒ No, all organizations use their own procedures
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**12.** Has your country set national prevention targets for 2025 for people who inject drugs?

- ☐ Yes  
☒ No

**12.1** If yes, what are the national prevention targets for 2025 for people who inject drugs?

**12.1.a** Annual number of new HIV infections among people who inject drugs: \_\_\_\_\_

**12.1.b** Percentage of people who inject drugs reached by prevention programmes: \_\_\_\_\_

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**13.** Do national policies exclude people who are currently using drugs from receiving antiretroviral therapy?

- ☐ Yes  
☒ No
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**14.** Are needle and syringe programmes operational in your country?

- ☐ Yes  
☐ No, not at all  
☒ No, but needles and syringes can be purchased legally in pharmacies without a prescription
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15. Can possession of a needle or syringe without a prescription be used as evidence of drug use or cause for arrest in your country?
- ☐ Yes
- ☒ No
- 
16. Are opioid agonist maintenance therapy programmes operational in your country?
- ☐ Yes
- ☒ No
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17. Is naloxone (used to reverse opioid overdoses) available through community distribution in your country?
- ☐ Yes
- ☒ No
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#### HIV prevention services for people in prisons and other closed settings

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18. Does your country have a national strategy that includes actions to reduce new infections among and provide services to people in prisons and other closed settings?
- ☒ Yes
- ☐ No
- 18.1 If yes, does the national strategy include for people in prisons and other closed settings (select all that apply):
- ☐ Community empowerment and capacity-building for organizations of people formerly in prisons or other closed settings
- ☐ Community in-reach services
- ☐ Distribution of condoms and condom-compatible lubricants
- ☒ Sexually transmitted infection prevention, screening and treatment services
- ☒ Psychosocial counselling and/or mental health services
- ☐ Hepatitis prevention, testing and treatment services
- ☒ Clinical services
- ☐ Legal support services
- ☐ Actions to address violence, including psychological first aid
- ☐ Post-exposure prophylaxis (PEP)
- ☒ Actions to reduce stigma and discrimination
- ☐ Needle and syringe programmes
- ☐ Opioid agonist maintenance therapy
- 
19. Are there national standard operating procedures (SOPs) or detailed national implementation guidelines for organizations implementing HIV prevention programmes for people in prisons and other closed settings?
- ☒ Yes, national SOPs apply for all organizations
- ☐ No, there are SOPs only for organizations under specific programmes
- ☐ No, all organizations use their own procedures
- 
20. Has your country set national prevention targets for 2025 for people in prisons and other closed settings?
- ☐ Yes
- ☒ No
- 20.1 If yes, what are the national prevention targets for 2025 for people in prisons and other closed settings?
- 20.1.a Annual number of new HIV infections among people in prisons and other closed settings: \_\_\_\_\_
- 20.1.b Percentage of people in prisons and other closed settings reached by prevention programmes: \_\_\_\_\_
- 
21. Are needle and syringe programmes operational in prisons and other closed settings in your country?
- ☐ Yes
- ☒ No
- 21.1 If yes, in what percentage of prisons and other closed settings?
- ☐ In few (<50%)
- ☐ In many (50–95%)
- ☐ Countrywide (>95%)
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**22. Are opioid agonist maintenance therapy programmes operational in prisons and other closed settings in your country?**

☐ Yes

☒ No

**22.1 If yes, in what percentage of prisons and other closed settings?**

☐ In few (<50%)

☐ In many (50–95%)

☐ Countrywide (>95%)

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**23. Are condoms and lubricants available to people in prisons and other closed settings in your country?**

☐ Yes

☒ No

**23.1 If yes, in what percentage of prisons and other closed settings?**

☐ In few (<50%)

☐ In many (50–95%)

☐ Countrywide (>95%)

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**24. Are HIV tests in prisons and other closed settings in your country:**

**24.a Carried out only with the informed consent of people in prisons and other closed settings?**

☒ Yes

☐ No

**24.b Systematically offered at entry and/or exit?**

☒ Yes

☐ No

**24.c Free of charge?**

☒ Yes

☐ No

**24.d Confidential?**

☒ Yes

☐ No

**24.e Available at any time during detention?**

☒ Yes

☐ No

**24.f Accompanied by relevant and accessible information?**

☒ Yes

☐ No

**24.g Accompanied by confidential pre- and post-test counselling?**

☒ Yes

☐ No

**24.h Equally accessible to all people in prisons and other closed settings?**

☒ Yes

☐ No

**24.h.i. If no to Q24.h, which people in prisons and other closed settings do not have equal access? \_\_\_\_\_**

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**25. Is antiretroviral therapy available to people in prisons and other closed settings living with HIV in your country?**

☒ Yes

☐ No

**25.1 If yes, in what percentage of prisons and other closed settings?**

☐ In few (<50%)

☐ In many (50–95%)

☒ Countrywide (>95%)

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26. Is hepatitis C treatment (using direct-acting antiviral medicines) available in prisons and other closed settings in your country?

☒ Yes

☐ No

26.1 If yes, in what percentage of prisons and other closed settings?

☐ In few (<50%)

☐ In many (50–95%)

☒ Countrywide (>95%)

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27. Are there post-release programmes to ensure continuity of care for:

27.a. Opioid agonist maintenance therapy?

☐ Yes

☒ No

27.b. Antiretroviral therapy?

☒ Yes

☐ No

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**HIV prevention among adolescent girls, young women and their male partners in communities with high HIV incidence**

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28. Does your country have a national strategy that includes actions to reduce new HIV infections among adolescent girls, young women and their male partners in communities with high HIV incidence?

☒ Yes

☐ No

☐ Not applicable

28.1 If yes, does the national strategy include (select all that apply):

- ☒ Community-based outreach
- ☒ Community-based promotion and distribution of condoms specifically for these populations
- ☒ Youth-friendly health services
- ☒ School-based HIV prevention campaigns (in addition to or as part of sexuality education)
- ☒ New media interventions
- ☒ Social support/economic empowerment

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29. Has your country set national HIV prevention targets for adolescent girls, young women and their male partners in communities with high HIV incidence for 2025?

☐ Yes

☒ No **Targets have been set for young people 16 - 24 years but not disaggregated by sex**

29.1 If yes, what are the national prevention targets for 2025 for adolescent girls and young women and their male partners?

29.1.a Annual number of new HIV infections among adolescent girls and young women aged 15–29 years (please specify number): \_\_\_\_\_

29.1.b Percentage of young women (aged 15–29 years) in high-incidence communities reached by HIV prevention services: \_\_\_\_\_

29.1.c Percentage of male partners (aged 20–34 years) of adolescent girls and young women in high-incidence communities reached by HIV prevention services: \_\_\_\_\_

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## Pre-exposure prophylaxis

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**30. Do your country's national guidelines recommend any of the following pre-exposure prophylaxis (PrEP) modalities or products (select all that apply)?**

- ☒ Daily oral PrEP
- ☒ Event-driven (on demand) PrEP
- ☐ The dapivirine vaginal ring (DPV-VR)
- ☐ Long-acting Injectable cabotegravir (CAB-LA)
- ☐ No PrEP modalities/products are recommended in the national guidelines

**30.1 To which populations is PrEP provided under the national guidelines (select all that apply)?**

- ☒ Gay men and other men who have sex with men
- ☒ Sex workers
- ☐ People who inject drugs
- ☒ Transgender people
- ☒ Serodiscordant couples
- ☒ Young women (aged 18–24 years)
- ☒ Adolescents (aged <17 years)
- ☐ People in prisons and other closed settings
- ☐ Pregnant and breastfeeding women
- ☒ People who request PrEP
- ☐ Other (please specify): \_\_\_\_\_
- ☐ No national PrEP guidelines have been developed

**30.2 Who has the authority to prescribe pre-exposure prophylaxis (PrEP) in your country (select all that apply)?**

- ☒ Doctors
- ☐ Clinical officers
- ☐ Nursing cadre (e.g. midwives, nurse practitioners, registered nurses)
- ☐ Pharmacists
- ☐ Other (please specify): \_\_\_\_\_

**30.3 Is PrEP available through any of the following in your country(select all that apply)?**

- ☒ Public health-care facilities
  - ☒ Community-based distribution (including mobile services)
  - ☐ Pharmacies (stand-alone, including online)
  - ☒ Private health-care providers
  - ☐ The Internet (informal purchases)
  - ☐ Educational institutions (e.g. schools, universities)
  - ☐ Research sites
  - ☐ Other (please specify): \_\_\_\_\_
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### Voluntary medical male circumcision

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Please note that these questions are only asked of 15 countries with high HIV prevalence, low levels of male circumcision and generalized heterosexual epidemics: Botswana, Eswatini, Ethiopia, Kenya, Lesotho, Malawi, Mozambique, Namibia, Rwanda, South Africa, South Sudan, Uganda, the United Republic of Tanzania, Zambia and Zimbabwe.

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**31. Does your country have a national strategy for voluntary medical male circumcision (VMMC)?**

- ☐ Yes  
☐ No

**31.1 If yes, does it refer to a specific age group?**

- ☐ Yes, it refers to a specific age group (please specify): \_\_\_\_\_  
☐ No age group is specified

**31.1.a If the national voluntary medical male circumcision (VMMC) strategy refers to a specific age group, has a target been set for the number of men in this age group to be reached?**

- ☐ Yes  
☐ No

**31.1.a.i. If yes, please specify:**

- ☐ The target number of men in that age group: \_\_\_\_\_  
☐ The target year: \_\_\_\_\_

**31.2 If yes to Q31, does the strategy include any of the following (select all that apply)?**

- ☐ Guidance on obtaining informed consent from clients  
☐ Guidance on filing client records, including informed consent forms, to ensure client confidentiality  
☐ Guidance on avoiding coercion

**31.3 If yes to Q31, does the strategy recommend the delivery of voluntary medical male circumcision (VMMC) alongside any of the following (select all that apply)?**

- ☐ Safer sex education  
☐ Condom education and promotion  
☐ HIV testing services  
☐ Sexually transmitted infection management, including referral

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**32. What medical male circumcision methods are recommended/approved by the national programme (select all that apply)?**

- ☐ Conventional surgical methods (dorsal slit, forceps guided, sleeve resection)  
☐ World Health Organization (WHO) prequalified device method approved for use

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**33. Does your country have a voluntary medical male circumcision (VMMC) sustainability strategy?**

- ☐ Yes  
☐ No
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## Condoms

- 34.** Does your country have a strategy or plan related to condoms (standalone or specific)?
- ☒ Yes
- ☐ No
- 34.1** If yes, does the national condom strategy or plan explicitly address the needs of (and targeted condom programming for) the following populations (select all that apply)?
- ☒ People living with HIV
- ☒ Sex workers (male and female)
- ☒ Gay men and other men who have sex with men
- ☐ People who inject drugs
- ☒ Young people (aged 15–24 years)
- ☒ People with sexually transmitted infections
- ☐ People in prisons and other closed settings
- ☒ The general public
- 34.2** If yes, does the national condom strategy or plan include any of the following (select all that apply)?
- ☒ Free distribution to key populations
- ☐ Subsidized condom marketing
- ☒ Private sector sales
- ☒ Condom promotion through mass media
- ☒ Condom promotion in (secondary) schools
- ☐ Condom distribution in (secondary) schools
- ☒ Condom promotion through outreach to communities with high HIV incidence
- 35.** Has the national need for condoms been estimated?
- ☒ Yes
- ☐ No
- 35.1** If yes, what is the estimated number of condoms needed? 18.7416 M
- 35.2** If yes, for what year is the condom needs estimate? 2017
- 35.3** If yes, what method was used to estimate the number of condoms needed (select all that apply)?
- ☒ General population (condoms per sexually active man/year)
- ☐ Historical (same as last year + population growth)
- ☐ Budget-driven (based on what can be bought)
- ☒ Demand-based (based on past condom usage rates, such as using the GOALS model)
- ☐ Capacity-based (how many can be supplied and distributed with current capacity)
- ☐ Part of family planning commodity needs estimates
- ☐ "Total universe of need" approach
- ☐ UNFPA/UNAIDS Condom Needs and Resource Requirement Estimation Tool
- ☐ Other (please specify): \_\_\_\_\_
- 36.** Can the possession of condoms be used as sufficient evidence of arrest or to support the prosecution of a crime in your country?
- ☐ Yes
- ☒ No
- 37.** Are there age restrictions for accessing condoms in your country?
- ☒ Yes
- ☐ No
- 38.** Are there restrictions on distributing condoms in public places in your country?
- ☐ Yes (please specify): \_\_\_\_\_
- ☒ No

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39. **Is there a policy or guidance for provider-initiated condom promotion and distribution in place (i.e., for a health service provider actively offering condom counselling and a condom supply)?**

- ☒ Yes, for all people of reproductive age visiting a health facility
  - ☒ Yes, for all key populations and other people at higher risk of HIV infection
  - ☒ Yes, for all young people who are sexually active
  - ☒ Yes, for all people accessing HIV testing services
  - ☒ Yes, for all people accessing hormonal or other methods of contraception
  - ☒ Yes, for all serodiscordant couples
  - ☐ No guidance in place
- 

40. **Have there been condom stock-outs\* in the past 12 months?**

40.a **National stock-outs**

- ☐ Yes
- ☒ No

40.b **Local stock-outs**

- ☒ Yes
  - ☐ No
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#### Services for people affected by humanitarian emergencies

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**41.** Are the following services accessible to people affected by humanitarian emergencies in your country? This includes all people affected by emergencies, including but not limited to nondisplaced people, refugees and asylum seekers, internally displaced people and migrants.

**41.a** HIV counselling and testing

- ☒ Yes  
☐ No  
☐ Not applicable

**41.b** Preventing vertical HIV transmission<sup>1</sup>

- ☒ Yes  
☐ No  
☐ Not applicable

**41.c** HIV treatment

- ☒ Yes  
☐ No  
☐ Not applicable

**41.d** Tuberculosis (TB) screening and treatment

- ☒ Yes  
☐ No  
☐ Not applicable

**41.e** Preventing and treating sexually transmitted infections

- ☒ Yes  
☐ No  
☐ Not applicable

**41.f** Services for key populations

- ☒ Yes  
☐ No  
☐ Not applicable

**41.g** Services for survivors of sexual and gender-based violence\*

- ☒ Yes  
☐ No  
☐ Not applicable

**41.h** Food and nutrition support

- ☒ Yes  
☐ No  
☐ Not applicable
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<sup>1</sup> In this document, vertical transmission includes transmission to the child that occurs during pregnancy, delivery or breastfeeding. "Vertical transmission" in this document is used as a neutral, non-stigmatizing alternative to "mother-to-child" transmission.

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### Refugees and asylum seekers

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42. Do national laws/policies enable refugees and asylum seekers to access HIV services (testing, antiretroviral medicine and care) under the same conditions as citizens?

42.a Refugees

☒ Yes

☐ No

42.a.i If yes, has the policy been implemented?

☒ Yes

☐ No

42.b Asylum seekers

☒ Yes

☐ No

42.b.i If yes, has the policy been implemented?

☒ Yes

☐ No

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### Migrant populations<sup>2</sup>

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43. Do national laws/policies enable migrants to access HIV services (testing, antiretroviral medicine and care) under the same conditions as citizens?

43.a Documented migrants

☒ Yes

☐ No

43.a.i If yes, has the policy been implemented?

☒ Yes

☐ No

43.b Undocumented migrants

☒ Yes

☐ No

43.b.i If yes, has the policy been implemented?

☒ Yes

☐ No

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### Comprehensive sexuality education

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44. Does your country have education policies that guide the delivery of life skills-based HIV and sexuality education\*, according to international standards\*, in:

44.a Primary school?

☒ Yes

☐ No

44.b Secondary school?

☒ Yes

☐ No

44.c Teacher training?

☒ Yes

☐ No

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<sup>2</sup> Migrants are defined as non-citizens who are in a country other than their country of origin for a stay of longer than six months, excluding refugees and asylum seekers.

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45. **Are representatives of the Ministry of Education included in the national HIV coordinating mechanism or equivalent?**

☐ There is no national HIV coordinating mechanism or equivalent

☐ There is a national HIV coordinating mechanism or equivalent, but it does not include any representatives of the Ministry of Education

☒ There is a national HIV coordinating mechanism or equivalent, and it includes representatives of the Ministry of Education

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46. **Has your country set a national target on comprehensive knowledge of HIV among adolescents and young people?**

☒ Yes

☐ No

46.1 **If yes, is there a national mechanism in place to monitor and review progress towards the target?**

☒ Yes

☐ No

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47. **Is life skills-based HIV and sexuality education\* part of the national curriculum or equivalent, in:**

47.a **Primary school?**

☒ Yes

☐ No

47.b **Secondary school?**

☒ Yes

☐ No

47.c **Teacher training?**

☒ Yes

☐ No

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48. **Does the national Education Management Information System (EMIS) or other education sector monitoring tool include questions about delivery of the following components of life skills-based HIV and sexuality education\*?**

|                                                                                                                                                                                                 | Questions in EMIS or equivalent                                        | Data collected                                                         |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------|------------------------------------------------------------------------|
| 48.a <b>Generic life skills</b> (e.g. decision-making, communication and refusal skills)                                                                                                        | <input checked="" type="checkbox"/> Yes<br><input type="checkbox"/> No | <input checked="" type="checkbox"/> Yes<br><input type="checkbox"/> No |
| 48.b <b>Sexual and reproductive health/sexuality education</b> (e.g. teaching on human growth and development, family life, reproductive health, sexual abuse, sexually transmitted infections) | <input checked="" type="checkbox"/> Yes<br><input type="checkbox"/> No | <input checked="" type="checkbox"/> Yes<br><input type="checkbox"/> No |
| 48.c <b>HIV transmission and prevention</b>                                                                                                                                                     | <input type="checkbox"/> Yes<br><input checked="" type="checkbox"/> No | <input type="checkbox"/> Yes<br><input checked="" type="checkbox"/> No |

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## 2. 95–95–95 for HIV testing and treatment

- Reduce annual AIDS-related deaths to under 250 000 by 2025.
- Ensure that 34 million people are on HIV treatment by 2025.
- Achieve the 95–95–95 testing, treatment and viral suppression targets within all demographics, groups and geographic settings, including children and adolescents living with HIV:
  - 95% of people living with HIV know their HIV status.
  - 95% of people who know their HIV-positive status are accessing treatment.
  - 95% of people on treatment have suppressed viral loads.
- Ensure that 90% of people living with HIV receive preventive treatment for tuberculosis (TB) by 2025.
- Reduce TB-related deaths among people living with HIV by 80% by 2025 (compared to a 2010 baseline).

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### HIV testing

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**49. Which of the following HIV testing approaches are used in your country (select all that apply)?**

- ☒ Client-initiated testing and counselling
- ☒ Provider-initiated testing and counselling
- ☒ Indicator condition testing
- ☒ Routine antenatal testing
- ☒ Dual HIV/syphilis rapid diagnostic tests for pregnant women in antenatal care
- ☒ Dual HIV/syphilis rapid diagnostic tests for any key population group
- ☒ Community-based testing
- ☒ Lay provider testing
- ☒ Self-testing
- ☒ Provider-assisted referral (assisted partner notification/index testing)
- ☒ Social network-based HIV testing for key populations
- ☒ Social network-based HIV testing for general population
- ☐ Other (please specify): \_\_\_\_\_

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**50. Has your country adapted the recommendations from the 2019 World Health Organization (WHO) *Consolidated guidelines on HIV testing services* in a national process on testing guidelines?**

- ☒ Yes, fully
  - ☐ Yes, partially
  - ☐ No
  - ☐ Don't know
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51. **Has your country included HIV self-testing as a national policy (either within the national HIV testing policy/plan or a stand-alone HIV self-testing policy)?**

☒ Yes

☐ No

51.1 **If yes, is HIV self-testing routinely implemented in your country?**

☒ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in select districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

51.2 **If no to Q51, is a national policy or plan on HIV self-testing in development?**

☐ Yes, and self-testing is being piloted to inform policy

☐ Yes, but self-testing is not being piloted

☐ No

51.2a **If yes to Q51.2, please indicate the year in which a national self-testing policy or plan is expected to be completed.**

☐ No planned year

☐ 2024

☐ 2025

☐ 2026

☐ 2027

51.3 **If yes to Q51, has your country included HIV self-testing to support PrEP initiation or continuation (either within the national HIV testing policy or plan or as a standalone HIV self-testing policy)?**

☒ Yes

☐ No

51.3a **If yes to Q51.3, is HIV self-testing to support PrEP routinely implemented in your country?**

☒ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in selected districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

51.4 **If yes to Q51, has your country included HIV self-testing in facilities (either within the national HIV testing policy or plan or as a standalone HIV self-testing policy)?**

☐ Yes

☒ No

51.4a **If yes to Q51.4, is HIV self-testing in facilities routinely implemented in your country?**

☐ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in selected districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

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52. **Has your country included partner services (provider-assisted referral, assisted partner notification, index testing) as a national policy?**

☒ Yes

☐ No

52.1 **If yes, which partner services approaches are being used (select all that apply)?**

☒ Partner referral (client refers their partners for testing without provider assistance)

☒ Enhanced partner referral (client receives some help from a provider to refer their partners for testing)

☒ Delayed provider-assisted referral (provider contacts partners after an agreed time period)

☒ Assisted partner services (provider contacts partners for testing)

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**53.** Has your country included social network-based HIV testing service approaches as a national policy?

☒ Yes

☐ No

**53.1** If yes to Q53, please identify the social network approaches that are available in your country (select all that apply):

☒ Referral-based

☒ Coupon-based

☒ Secondary distribution of HIV self-testing

**53.1a** If yes to Q53, in which populations are social network-based testing approaches being used (select all that apply)?

☒ Key populations

☒ General population

---

**54.** Has your country adopted or included dual HIV/syphilis rapid diagnostic tests for pregnant women and/or key populations as a national policy or plan?

☐ Yes, for pregnant women only

☐ Yes, for key populations only

☒ Yes, for both pregnant women and key populations

☐ No

**54.1** If yes, is dual HIV/syphilis rapid diagnostic testing routinely implemented in your country?

**54.1.a** For pregnant women:

☒ Yes, routinely implemented on a national scale

☐ Yes, routinely implemented on a subnational scale or in select districts

☐ No, only in pilot projects

☐ No, not implemented anywhere

**54.1.b** For people from key populations:

☒ Yes, routinely implemented on a national scale, including subnational

☐ Yes, routinely implemented on a subnational scale or in selected districts (not at a national scale)

☐ No, only in pilot projects

☐ No, not implemented anywhere

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**55.** Does your country use three consecutive reactive tests (3-test strategy/algorithm) for an HIV-positive diagnosis ?

☐ Yes

☒ No

**55.1** If no, does your country have a plan to adopt a 3-test strategy/algorithm for an HIV-positive diagnosis?

☐ Yes

☒ No

**55.2** If yes, please indicate the year in which a national policy on 3-test strategy for an HIV-positive diagnosis is planned to be adopted.

☐ No planned year

☐ 2024

☐ 2025

☐ 2026

☐ 2027

**55.3** Does your country use the following assays in the standard/routine national testing strategy/algorithm?

☒ Rapid diagnostic test

☒ Immunoassay

☐ Western blotting

☒ Nucleic acid test

☐ Recency test

**55.4** Does your country have a policy for systematic retesting before antiretroviral therapy initiation?

☐ Yes

☒ No

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56. Does your country have a policy specifying that HIV testing will be provided?

☒ Free to all

☐ Free to some

☐ At a cost

☐ No policy on HIV testing cost

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57. Is there a law, regulation or policy specifying that HIV testing:

57.a Is mandatory before marriage?

☐ Yes

☒ No

57.b Is mandatory to obtain a work or residence permit?

☐ Yes

☒ No

57.c Is mandatory for certain groups or professions (not including blood and/or tissue donors)?

☒ Yes

☐ No

57.c.i. If yes, please specify which groups: **Law Enforcement (Jamaica Constabulary Force & Jamaica Defense Force)**

---

58. Does your country have national policies and/or strategies on linking HIV testing and counselling and enrolment in care following an HIV-positive diagnosis?

☒ Yes

☐ No

58.1 If yes, what do they include (select all that apply)?

☒ Streamlined interventions (i.e., enhanced linkage, disclosure and tracing)

☒ Peer support and patient navigation approaches

☒ Psychosocial interventions, including peer support and social networks for adolescents living with HIV

☒ Quality improvement approaches

☒ Home/community-based antiretroviral therapy initiation

☒ Use of CD4 testing as screening tool for linking to an advanced HIV disease care package

☒ Co-located facility-based HIV testing and antiretroviral therapy services

☒ Tuberculosis–HIV collaborative activities other than co-location

☐ Compensation or incentives

☒ Phone call or SMS/text message

☒ Virtual interventions including digital follow-up via social media or other online platform

☐ Others (please specify): \_\_\_\_\_

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59. Does your country have national policies and/or strategies on linking HIV testing and prevention following an HIV-negative diagnosis?

☐ Yes, for all populations

☒ Yes, but only for key populations and high-risk groups

☐ No

59.1 If yes, what do they include (select all that apply)?

☐ Streamlined interventions (enhanced linkage or disclosure)

☒ Peer support and patient navigation approaches

☐ Quality improvement approaches

☒ Co-located facility-based HIV testing and pre-exposure prophylaxis (PrEP) services

☐ Co-located facility-based HIV testing and voluntary medical male circumcision (VMMC) services

☐ Compensation or incentives

☐ Phone call or SMS/text message

☒ Virtual interventions including digital follow-up via social media or other online platform

☐ Others (please specify): \_\_\_\_\_

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### Antiretroviral therapy

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**60. Has your country adopted the recommendations on rapid Initiation of antiretroviral therapy in the 2021 World Health Organization (WHO) Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach?**

- ☒ Yes, rapid initiation within 7 days of HIV diagnosis  
☐ No  
☐ Other (please specify): \_\_\_\_\_

**60.1 If your country has adopted a policy on rapid initiation of antiretroviral therapy, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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**61 Does your country have a policy to offer starting antiretroviral therapy on the same day as an HIV diagnosis, as part of the policy on rapid Initiation of antiretroviral therapy?**

- ☒ Yes  
☐ No

**61.1 If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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**62. Is CD4 testing for diagnosing advanced HIV disease available?**

- ☒ Yes  
☐ No

**62.1 Is yes, where is it available (select all that apply)?**

- ☒ Point-of-care  
☒ Facility laboratory  
☒ Centralized laboratory  
☐ Other (please specify): \_\_\_\_\_

**62.2 If yes, in what percentage of sites (estimated) do clients have access to CD4 testing and return of results?**

- ☐ In few (<50%) sites  
☐ In many (50–95%) sites  
☒ Countrywide (>95% of sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

**62.3 If yes, what is the median time (in number of days) for the person to receive the CD4 result?**

- ☒ Please specify: 1 week  
☐ Not available

---

**63. Is nurse-initiated antiretroviral therapy allowed in your country for any of the following populations (select all that apply)?**

- ☐ Adults, except pregnant women  
☐ Pregnant women  
☐ Adolescents (aged 10–19 years)  
☐ Children younger than 10 years  
☒ None of the above
-

---

**64.** Does your country have a national policy promoting community delivery of antiretroviral therapy (such as outside of health facilities)?

☐ Yes

☒ No

**64.1** If yes, where is delivery in a community setting implemented?

- ☐ Nationally
- ☐ Regionally
- ☐ At pilot sites
- ☐ Other (please specify): \_\_\_\_\_

**64.2** If yes, to which populations is antiretroviral therapy provided in community settings in your country (such as outside of health facilities)?

- ☐ For all people on antiretroviral therapy, including pregnant and breastfeeding women and children
- ☐ For all people on antiretroviral therapy, excluding pregnant and breastfeeding women and children
- ☐ For all people on antiretroviral therapy, including pregnant and breastfeeding women, but excluding children
- ☐ For all people on antiretroviral therapy, including children, but excluding pregnant and breastfeeding women
- ☐ For all people who are stable on antiretroviral therapy, according to the national guidelines
- ☐ Other (please specify): \_\_\_\_\_

**64.3** If yes, which differentiated care services is your country using for the pick-up of antiretroviral medicine (select all that apply)?

- ☐ Pharmacy pick-up at the same site as the health facility
- ☐ Other pharmacy (e.g. standalone)
- ☐ Adherence group at the same site as the health facility
- ☐ Community pick-up points (individual)
- ☐ Community-based adherence groups
- ☐ Other (please specify): \_\_\_\_\_

---

**65.** Does your country have a national policy on the frequency of clinic visits for adults who are established\* on antiretroviral therapy?

☒ Yes

☐ No

**65.1** If yes, please specify the frequency of clinic visits in the national policy.

- ☐ Once a month
- ☐ Every 2 months
- ☐ Every 3 months
- ☒ Every 6 months
- ☐ Every 12 months

**65.2** If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites
- ☐ Implemented in many (50–95%) treatment sites
- ☒ Implemented countrywide (>95% of treatment sites)
- ☐ Not implemented in practice
- ☐ Other (please specify): \_\_\_\_\_
-

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66. Does your country have a national policy on how frequently adults who are established\* on antiretroviral therapy should pick-up antiretroviral medicine?

☒ Yes  
☐ No

66.1 If yes, please specify the frequency of antiretroviral medicine pick-up included in the national policy.

☐ Once a month  
☐ Every 2 months  
☒ Every 3 months  
☐ Every 6 months  
☐ Every 12 months  
☐ Other (please specify): \_\_\_\_\_

66.2 If yes, what is the status of implementation?

☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

---

67. Do the country's national criteria for (or definition of) people established\* on antiretroviral therapy include the following elements defined in the 2021 World Health Organization (WHO) Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach (select all that apply)?

☒ Receiving antiretroviral therapy for at least 6 months  
☒ No current illness (does not include well-controlled chronic health conditions)  
☒ Good understanding of lifelong adherence  
☒ Evidence of treatment success (i.e. at least one suppressed viral load result within the past six months)  
☒ Other (please specify): **Two consecutive suppressed viral load results six months apart**

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68. Does the country provide psychological support for adolescents living with HIV?

☒ Yes  
☐ No

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69. Does your country implement interventions to trace people who have disengaged from care and provide support for reengagement?

☒ Yes  
☐ No

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70. Please provide the country's national criteria for (or definition of) lost to follow-up. For guidance, the World Health Organization (WHO) defines "lost to follow-up" as a patient who has not received antiretroviral medicines within 28 days of their last missed drug collection appointment.<sup>3</sup>

90 days since their last missed appointment

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71. Has your country adopted the recommendations in the 2021 World Health Organization (WHO) Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach to offer a package of interventions to all patients presenting with advanced HIV disease (defined by WHO as CD4<200)?

☒ Yes, fully adopted  
☐ Yes, partially adopted (only for specific interventions and/or populations, such as children, adolescents or adults) (please specify): \_\_\_\_\_  
☐ No

71.1 If yes, how widely is it implemented?

☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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3 Source: Consolidated guidelines on person-centred HIV strategic information: strengthening routine data for impact. Geneva: World Health Organization; 2022, page 127)

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72. Which of the following components of the package of advanced HIV disease interventions for tuberculosis (TB), severe bacterial infections and cryptococcal meningitis recommended in the 2021 WHO Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach are included in the national policy on antiretroviral therapy for adults, adolescents and children (select all that apply)?

- ☒ Baseline CD4 count test to diagnose advanced HIV disease
- ☒ Molecular diagnostic tests for TB diagnosis
- ☐ Urine LF-LAM for TB diagnosis
- ☐ Cryptococcal antigen (CrAg) screening
- ☒ Co-trimoxazole prophylaxis
- ☐ TB preventive treatment
- ☒ Fluconazole empirical prophylaxis
- ☐ Fluconazole pre-emptive therapy
- ☒ Rapid antiretroviral therapy initiation
- ☒ Adapted adherence support
- ☐ Other (please specify): \_\_\_\_\_

---

73. Which of the following service provision modalities are included in the national policy on antiretroviral therapy for adults, adolescents and children (select all that apply)?

- ☒ Tuberculosis (TB) service providers provide antiretroviral therapy in TB clinics for the duration of TB treatment
  - ☐ Antiretroviral therapy providers provide TB treatment in antiretroviral therapy settings for the duration of TB treatment
  - ☒ Maternal, newborn and child health (MNCH) service providers provide antiretroviral therapy in MNCH clinics
  - ☒ Antiretroviral therapy providers deliver antiretroviral therapy for pregnant women
  - ☒ Antiretroviral therapy providers deliver antiretroviral therapy for newborns, infants and children
  - ☒ Nutrition assessment, counselling and support provided to malnourished people living with HIV
  - ☐ Antiretroviral therapy delivered in settings providing opioid agonist maintenance therapy
  - ☒ Primary health-care providers deliver antiretroviral therapy in primary health care for adults and adolescents
  - ☒ Primary health-care providers deliver antiretroviral therapy in primary health-care settings for children
  - ☒ Psychosocial support strategies for patient-centered care (e.g. support groups, enhanced adherence counselling, support for disclosure or referral for psychological/socioeconomic services) linked to facilities
  - ☒ Patient-centered support (e.g. counselling, enhanced adherence counselling, support for disclosure or referral for psychological/socioeconomic services) separated from facilities
  - ☒ Key population-friendly services
  - ☒ Adolescent-friendly health services
  - ☒ Antiretroviral therapy delivered in the community as part of a differentiated care model
  - ☒ Antiretroviral therapy providers carry out cardiovascular disease screening and management
  - ☒ Antiretroviral therapy providers carry out mental health screening and treatment
  - ☐ Other (please specify): \_\_\_\_\_
-

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**74.** Do patients pay any routine user fees or charges for services when visiting a public sector health facility?

- ☐ Yes  
☒ No

**74.1** If yes, is there a specific formal fee or an informal/variable fee for the following?

**74.1.a.** HIV testing

- ☐ Formal  
☐ Informal

**74.1.b.** Dispensing of pre-exposure prophylaxis (PrEP)

- ☐ Formal  
☐ Informal

**74.1.c.** Primary care appointment

- ☐ Formal  
☐ Informal

**74.1.d.** Patient cards

- ☐ Formal  
☐ Informal

**74.1.e.** Diagnostic services (including viral load test)

- ☐ Formal  
☐ Informal

**74.1.f.** Dispensing of HIV treatment

- ☐ Formal  
☐ Informal

**74.1.g** Dispensing of prevention and treatment of co-infections

- ☐ Formal  
☐ Informal

**74.1.h** Dispensing of other co-therapies (eg: medicines for NCDs, sexual & reproductive health, immunizations)

- ☐ Formal  
☐ Informal
-

## Antiretroviral therapy regimens

### Adults and adolescents

**75.** Based on the recommendations in the 2021 WHO Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach, is TDF + 3TC (or FTC) + DTG the preferred first-line antiretroviral combination for treatment initiation in national guidelines for the following:

**75.a** Adults and adolescents?

- ☒ Yes  
☐ No, TDF + 3TC (or FTC) + DTG is included as alternative first-line regimen  
☐ No

**75.a.i.** If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

**75.a.ii** If no, what is (are) the preferred option(s):

- ☐ TDF + 3TC (or FTC) + EFV 600 mg  
☐ TDF + 3TC + EFV 400 mg  
☐ ABC + 3TC + DTG  
☐ TAF + 3TC (or FTC) + DTG  
☐ Other regimens (please specify): \_\_\_\_\_

**75.a.iii** If no, is there a plan to adopt TDF + 3TC (or FTC) + DTG as the preferred first-line antiretroviral combination for treatment initiation in 2024?

- ☐ Yes  
☐ No

**75.b** Women of childbearing age?

- ☒ Yes  
☐ No

**75.b.i** If no, what is (are) the preferred option(s):

- ☐ TDF + 3TC (or FTC) + EFV 600 mg  
☐ TDF + 3TC + EFV 400 mg  
☐ TAF + 3TC (or FTC) + DTG  
☐ Other regimens (please specify): \_\_\_\_\_

**75.b.ii** If no, is there a plan to adopt TDF + 3TC or (FTC) + DTG as the preferred first-line antiretroviral medicine combination for treatment initiation in 2024?

- ☐ Yes  
☐ No

**75.c.** Pregnant and/or breastfeeding women?

- ☒ Yes  
☐ No

**75.c.i** If no, what is (are) the preferred option(s):

- TDF + 3TC (or FTC) + EFV 600 mg  
TDF + 3TC + EFV 400 mg  
TAF + 3TC (or FTC) + DTG  
Other regimens (please specify): \_\_\_\_\_

**75.c.ii.** If no, is there a plan to adopt TDF + 3TC or (FTC) + DTG as the preferred first-line antiretroviral medicine combination for treatment initiation in 2024?

- ☐ Yes  
☐ No

---

76. Does your country use fixed-dose (FDC) antiretroviral therapy combinations as the preferred first-line therapy (select all that apply)?

☒ Yes, 3-drug fixed-dose combination, taken once a day

☐ Yes, 2-drug fixed-dose combination plus 1 other drug, taken once a day

☐ No

☐ Other (please specify): \_\_\_\_\_

---

77. Is a DTG-based regimen included in the national guidelines as an option for second-line antiretroviral combination for adults and adolescents with HIV?

☐ Yes, as preferred option

☐ Yes, as alternative option

☐ No

☒ Other (please specify): **Adolescents second line option**

---

78. Is an ATV/r-based (or LPV/r-based) regimen Included In the national guidelines as an option for second-line antiretroviral combination for adults and adolescents with HIV?

☒ Yes, as preferred option

☐ Yes, as alternative option

☐ No

☐ Other (please specify): \_\_\_\_\_

---

79. Is a darunavir/ritonavir-based regimen included in the national guidelines as an option for antiretroviral therapy of adults and adolescents with HIV?

☐ Yes, as preferred second-line option

☐ Yes, as alternative second-line option

☒ Yes, only as third-line option

☐ Not recommended

☐ Other (please specify): \_\_\_\_\_

---

## Children

- 80.** Are DTG regimens the preferred treatment initiation option in the national guidelines for all infants and children with HIV?
- ☒ Yes, for all children older than 4 weeks and weighing more than 3kg
- ☐ Yes, but only for children weighing more than 20kg
- ☐ No
- ☐ Other (please specify): \_\_\_\_\_
- 80.1** If DTG is not the preferred treatment option for infants and children older than 4 weeks and weighing more than 3kg, are LPV/r-based regimens the preferred treatment option?
- ☐ Yes, for all
- ☐ No, but only for children weighing less than 20kg
- ☐ No
- 
- 81.** What is the recommended NRTI\* backbone in the national guidelines for treatment initiation in children?
- ☐ TDF + 3TC (or FTC)
- ☐ AZT + 3TC (or FTC)
- ☒ ABC + 3TC (or FTC)
- ☐ Other (please specify): \_\_\_\_\_
- 
- 82.** Is DTG recommended as the preferred second-line option for children failing NNRTI\*-based regimens?
- ☐ Yes, for all children older than 4 weeks
- ☒ Yes, for children weighing more than 20 kg
- ☐ No
- ☐ Other (please specify): \_\_\_\_\_
- 
- 83.** Is DTG recommended as the preferred second-line option for children failing protease inhibiting-based regimens?
- ☒ Yes, for all children older than 4 weeks and weighing more than 3kg
- ☐ Yes, but only for children weighing more than 20kg
- ☐ No
- ☐ Other (please specify): \_\_\_\_\_
- 
- 84.** What is the recommended second-line option for children failing DTG-based regimens?
- ☒ LPV/r
- ☐ ATV/r
- ☐ Other (please specify): \_\_\_\_\_
- 
- 85.** Is a darunavir/ritonavir (DRV/r)-based regimen included in the national guidelines as an option for antiretroviral therapy of children with HIV?
- ☐ Yes, as preferred second-line option
- ☐ Yes, as alternative second-line option
- ☐ Yes, only as third-line option
- ☒ Not recommended
- ☐ Other (please specify): \_\_\_\_\_
- 
- 86.** Are any of the following early childhood development activities integrated into HIV programmes (select all that apply)?
- ☐ Responsive caregiving
- ☐ Promote early learning
- ☒ Integrate caregiving and nutrition interventions
- ☒ Support maternal mental health
- ☐ None of the above
-

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## Viral load

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**287. Please identify the measured threshold in national treatment guidelines at which viral load suppression<sup>4</sup> in an individual is defined as suppressed:**

- ☒ ≤1000 copies/ml  
☐ ≤400 copies/ml  
☐ ≤200 copies/ml  
☐ ≤50 copies/ml  
☐ Some countries use ≤20 – which category should they cross? Add a note to guide that?  
Not detected by assay or sample type used Other (please specify): \_\_\_\_\_
- 

**288. Does your country have a current national policy on routine viral load testing\* for monitoring antiretroviral therapy?**

**288.a For adults and adolescents**

- ☒ Yes  
☐ No

**288.a. i If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

**288.b For children**

- ☒ Yes  
☐ No

**288.b. i If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_
- 

**289. Does your country have a current national policy on point-of-care viral load testing?**

- ☐ Yes  
☒ No

**289.1 If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☐ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_
- 

**290. Are dried blood spot specimens recommended in the national policy for viral load testing?**

- ☐ Yes  
☒ No  
☐ Other (please specify): \_\_\_\_\_

**290.1 If yes, what is the level of implementation?**

- ☐ Fully  
☐ Partially  
☐ Not implemented
- 

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<sup>4</sup> The viral suppression threshold for may differ from the threshold to determine treatment failure.

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291. Does the country have a policy to prioritize viral load testing in select populations and/or situations (e.g., pregnant women, infants and adolescents)?



Yes



No

291.1 If yes, for which populations is viral load testing prioritized? Please select all that apply.



Pregnant and breastfeeding women



Patients with advanced HIV disease



Patients suspected of failing treatment



Infants and children (aged 0–<10 years)



Adolescents (aged 10–19 years)



Other (please specify): \_\_\_\_\_

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## HIV drug resistance and toxicity monitoring<sup>5</sup>

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87. Does your country have a national plan to monitor HIV drug resistance?

☒ Yes

☐ No

87.1. If yes, please specify the years covered by the plan: 2022 - 2025

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88. In the past three years, has your country carried out HIV drug resistance (HIVDR) surveillance according to any of the following World Health Organization (WHO) protocols?

88.a Pre-treatment drug resistance (PDR) surveys<sup>6</sup>

☐ Yes

☐ No, but there is a plan to implement the PDR survey this year

☒ No, and there is no plan to implement the PDR survey this year

88.a.i. If yes, please specify the year the last PDR survey started: \_\_\_\_\_

88.b HIV drug resistance survey among individuals exposed to pre-exposure prophylaxis (PrEP) diagnosed with HIV infection

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

88.b.i. If yes, please specify the year the last survey started: \_\_\_\_\_

88.c Acquired drug resistance surveys among adults<sup>7</sup>

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

88.c.i. If yes, please specify the year the last survey started: \_\_\_\_\_

88.d Acquired drug resistance surveys among children

☐ Yes

☐ No, but there is a plan to implement the survey this year

☒ No, and there is no plan to implement the survey this year

88.d.i. If yes, please specify the year the last survey started: \_\_\_\_\_

88.e HIV drug resistance among infants (<18 months) using early infant diagnosis<sup>8</sup>

☐ Yes

☐ No, but there is a plan to implement the infant survey this year

☒ No, and there is no plan to implement the infant survey this year

88.e.i. If yes, please specify the year the last infant survey started: \_\_\_\_\_

88.f Survey or routine monitoring of clinic performance using early warning indicators (EWI) for HIV drug resistance

☒ Yes

☐ No

88.f.i. If yes, please specify:

☐ Year it was last monitored: 2020

☐ Number of clinics monitored: 38

88.g The early warning indicators (EWI) for HIV drug resistance were collected through:

☐ EWI survey in a sample of clinics

☒ Routine patient monitoring systems

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5 Data from HIV drug resistance surveys should be routinely uploaded to the WHO HIVDR database. Ministry of Health/Antiretroviral Therapy Programme designated users can request access by contacting [hiv-aids@who.int](mailto:hiv-aids@who.int). For details, please see: <https://www.who.int/teams/global-hiv-hepatitis-and-stis-programmes/hiv/treatment/hiv-drug-resistance/hiv-drug-resistance-surveillance>

6 For more details, please see: Surveillance of HIV drug resistance in adults initiating antiretroviral therapy. Geneva: WHO; 2014 (<https://www.who.int/publications/i/item/9789241507196>).

7 For more details, please see: Surveillance of HIV drug resistance in adults receiving ART. Geneva: WHO; 2014 (<https://www.who.int/publications/i/item/9789241507073>).

8 For more details, please see: HIV drug resistance. In: World Health Organization: Global HIV Programme: Treatment & Care [website]. Geneva: WHO; c2018 (<https://www.who.int/teams/global-hiv-hepatitis-and-stis-programmes/hiv/treatment/hiv-drug-resistance>).

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89. Does your country have a national policy for HIV drug resistance testing for individual patients who fail antiretroviral therapy for clinical management?

☒ Yes

☐ No

89.1 If yes, which of the following groups are considered in the policy (select all that apply)?

- ☐ Adults failing DTG-based first-line antiretroviral therapy
- ☐ Children failing DTG-based first-line antiretroviral therapy
- ☒ Adults failing DTG-based second-line antiretroviral therapy
- ☒ Children failing DTG-based second-line antiretroviral therapy
- ☒ Adults failing protease inhibitor-based second-line antiretroviral therapy
- ☒ Children failing protease inhibitor-based second-line antiretroviral therapy
- ☒ Patients failing third-line antiretroviral therapy
- ☐ Other (please specify): \_\_\_\_\_

90. Does your country have a national policy for HIV drug resistance testing for individual patients who initiate antiretroviral therapy?

- ☐ Yes
- ☒ No

90.1 If yes, which of the following groups are considered in the policy (select all that apply)?

- ☐ All adults initiating first-line DTG-based first-line antiretroviral therapy
- ☐ People with a history of tenofovir-based PrEP for the prevention of HIV
- ☐ People with a prior history of cabotegravir-based PrEP for the prevention of HIV
- ☐ Infants and children initiating first-line antiretroviral therapy
- ☐ Pregnant women initiating first-line antiretroviral therapy
- ☐ Other (please specify): \_\_\_\_\_

---

91. Excluding passive pharmacovigilance approaches, does your country make an ongoing systematic effort to monitor the toxicity of antiretroviral medicines in the country?

- ☐ Yes
- ☒ No

91.1 If yes, what approaches are used (select all that apply)?

- ☐ Routine toxicity monitoring as part of the national monitoring and evaluation system
- ☐ Active toxicity monitoring/surveillance within cohorts in adults
- ☐ Active toxicity monitoring/surveillance within cohorts in adolescents and children
- ☐ Pregnancy registry and surveillance of birth defects

---

92. Have toxicity monitoring approaches been introduced to monitor adverse drug reactions to DTG use?

- ☒ Yes
- ☐ No

92.1 If yes, what approaches are used (select all that apply)?

- ☒ Routine toxicity monitoring as part of the national monitoring and evaluation system
- ☐ Active toxicity monitoring/surveillance within cohorts in adults
- ☐ Active toxicity monitoring/surveillance within cohorts in adolescents and children
- ☐ Pregnancy registry and surveillance of birth defects

92.2 If yes, has training of health-care workers on the management, capture and reporting of adverse drug reactions related to DTG been implemented?

- ☒ Yes
- ☐ No
-

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#### Adherence and retention

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**93.** Does your country have national policies and/or strategies on adherence support (community and facility-based)?

- ☒ Yes  
☐ No
- 

**94.** Are any of the following adherence support services being implemented in your country (select all that apply)?

- ☒ Peer counsellors  
☒ Text messages  
☒ Use of reminder devices  
☒ Patient reintegration follow-up calls/home visits  
☒ Enhanced adherence counselling  
☒ Referral to psychological/socioeconomic support  
☒ Cognitive behavioural therapy  
☒ Behavioural skills training/medication adherence training  
☒ Fixed-dose combinations and once-daily regimens  
☒ Case management  
☒ Peer navigation  
☐ Other (please specify): \_\_\_\_\_
- 

**95.** Does your country have national policies and/or strategies on retention in antiretroviral therapy?

- ☒ Yes  
☐ No
- 

**96.** Are any of the following retention support services being implemented in your country (select all that apply)?

- ☒ Community-based interventions  
☒ Adherence clubs and peer support  
☐ Other (please specify): \_\_\_\_\_
- 

**97.** Are treatment literacy programmes available in your country to people living with HIV, including information on side effects, drug resistance, etc.?

- ☒ Yes  
☐ No
-

98. Are the following screening tools recommended for people living with HIV in national guidelines related to tuberculosis (TB) and/or HIV (select all that apply)?

- ☒ World Health Organization-recommended four symptom screen for adults and adolescents (>10 years)
- ☐ C-reactive protein (CRP) for adults and adolescents (>10 years)
- ☐ Chest X-ray for adults and adolescents (>10 years)
- ☐ Molecular World Health Organization-approved rapid diagnostic tests for TB (mWRD) for adults and adolescents (>10 years)
- ☒ Symptom screen, including cough, fever, poor weight gain or close contact with a TB patient for children <10 years
- ☐ None of the above

99. Has your country adopted the 2019 WHO policy update on the use of lateral flow urine lipoarabinomannan assay (LF-LAM) for the diagnosis and screening of active tuberculosis in people living with HIV (select all age groups that apply)?

- ☐ Yes, for adults and adolescents (>10 years)
- ☐ Yes, for children (<10 years)
- ☒ No

100. Which of the following regimens are recommended for tuberculosis (TB) preventive treatment in national guidelines (select all that apply)?

100.a Adults and adolescents living with HIV

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 36 months of daily isoniazid monotherapy (36H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3RH)
- ☐ 1 month of daily rifapentine plus isoniazid (1HP)
- ☐ Other (please specify): \_\_\_\_\_
- ☒ None of the World Health Organization recommended TB preventive treatment regimens are recommended in national guidelines for adults and adolescents

100.a.i If more than one regimen is recommended, which is the preferred regimen?

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 36 months of daily isoniazid monotherapy (36H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3RH)
- ☐ 1 month of daily rifapentine plus isoniazid (1HP)
- ☐ Other (please specify): \_\_\_\_\_

100.b Children living with HIV

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3RH)
- ☐ Other (please specify): \_\_\_\_\_
- ☒ TB preventive treatment not recommended in national guidelines for children

100.b.i. If more than one regimen is recommended, which is the preferred regimen?

- ☐ 6 months of daily isoniazid monotherapy (6H)
- ☐ 9 months of daily isoniazid monotherapy (9H)
- ☐ 4 months of daily rifampicin (4R)
- ☐ 3 months of weekly rifapentine plus isoniazid (3HP)
- ☐ 3 months of daily rifampicin plus isoniazid (3RH)
- ☐ Other (please specify): \_\_\_\_\_

---

101. Are the following required in national guidelines prior to initiating tuberculosis (TB) preventive treatment in people with HIV?

101.a Skin tests or interferon-gamma release assay (IGRA) **n/a TB preventive therapy not in national guidelines**

☐ Yes, for all

☐ No

☐ Only if available

101.b X-ray

☐ Yes, for all

☐ No

☐ Only if available

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102. In the last reporting period, has there been a stock-out\* of any of the following?

102.a Isoniazid

☐ Yes, at the national level

☐ Yes, at the local level

☒ No

102.b Vitamin B6 **n/a**

☐ Yes, at the national level

☐ Yes, at the local level

☐ No

102.c Rifapentine (including fixed-dose combinations with isoniazid)

☐ Yes, at the national level

☐ Yes, at the local level

☒ No

---

103. What is the status of integration of the following HIV and tuberculosis (TB) services?

103.a World Health Organization-recommended rapid molecular diagnostics (e.g. Xpert MTB/RIF) are co-located:

☒ In few (<50%) health facilities providing HIV testing and care

☐ In many (50–95%) health facilities providing HIV testing and care

☐ Countrywide (>95% of health facilities providing HIV testing and care)

☐ Not integrated in practice

☐ Other (please specify): \_\_\_\_\_

103.b People living with HIV who have tuberculosis (TB) received antiretroviral medicines at the same place as they receive their TB treatment:

☐ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☒ Countrywide (>95% of health facilities)

☐ Not integrated in practice

☐ Other (please specify): \_\_\_\_\_

103.c Antiretroviral therapy is initiated by the same health-care worker providing tuberculosis (TB) treatment for people living with HIV who have TB:

☐ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☒ countrywide (>95% of health facilities)

☐ Not integrated in practice

☐ Other (please specify): \_\_\_\_\_

103.d Antiretroviral therapy and tuberculosis (TB) treatment for people living with HIV who have TB are monitored by one health-care worker:

☐ In few (<50%) health facilities

☐ In many (50–95%) health facilities

☐ Countrywide (>95%) of health facilities

☐ Not integrated in practice

☒ Other (please specify): **Initially for the intensive phase but in the continuation phase there is integration within the community**

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### 3. End paediatric AIDS and eliminate vertical transmission

- Ensure that 75% of all children living with HIV have suppressed viral loads by 2023 and 86% by 2025, in line with the 95–95–95 HIV treatment targets.
- Ensure that 95% of pregnant women have access to testing for HIV, syphilis, hepatitis B and other sexually transmitted infections by 2025.
- Ensure that 95% of pregnant and breastfeeding women in high HIV burden settings have access to retesting during late pregnancy and in the post-partum period by 2025.
- Ensure that all pregnant and breastfeeding women living with HIV are receiving life-long antiretroviral therapy, with 95% achieving and sustaining viral suppression before delivery and during breastfeeding by 2025.
- Ensure that all HIV-negative pregnant and breastfeeding women in high HIV burden settings—or those who have male partners at high risk of HIV in all settings—have access to combination prevention, including pre-exposure prophylaxis (PREP), and that 90% of their male partners who are living with HIV are continuously receiving antiretroviral therapy.
- Ensure that 95% of HIV-exposed children are tested by two months of age and after the cessation of breastfeeding.

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#### Prevention of vertical transmission of HIV

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**104.** Does your country have a policy on retesting HIV-negative women during pregnancy, delivery and/or the post-partum/breastfeeding period?  
☒ Yes  
☐ No

**104.1** If yes, when is retesting done?

**104.1.a** During pregnancy

☒ Yes  
☐ No

**104.1.a.i** If yes, which month of pregnancy: Third Trimester

**104.1.b** At delivery

☐ Yes  
☒ No

**104.1.c** Post-partum/breastfeeding

☐ Yes  
☒ No

**104.1.c.i** If yes, how long after delivery (in months): \_\_\_\_\_

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**105.** Does your country have a national plan for the elimination of vertical transmission of HIV:

☒ Yes  
☐ No

**105.1** If yes, please specify:

**105.1.a** Target(s) for the vertical transmission rate (%): < 2.0%

**105.1.b** Year: 2023

**105.1.c** Elimination target(s) (such as the number of cases/100 000 population): < 0.3 per 1000 live births

**105.1.d** Year: 2023

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**106.** Is your country implementing a treat all policy for pregnant and breastfeeding women living with HIV?

☒ Yes  
☐ No

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107. What is the current nationally recommended regimen for HIV-exposed infants for preventing the vertical transmission of HIV?  
☐ Please specify the infant prophylaxis regimen: Nevirapine 2mg/kg immediately and Zidovudine 4mg/kg  
☐ Recommended duration of the regimen: 6 weeks

107.a Are different regimens recommended for high-risk infants?  
☒ Yes  
☐ No

107.a.i If yes, please specify the regimens: Nevirapine for 6 weeks and Zidovudine for 6 weeks

107.a.ii What is the definition of "high-risk infant" in the national policy (select all that apply)?  
☒ Born to women with established HIV infection who have received less than 4 weeks of antiretroviral therapy at the time of delivery  
☒ Born to women with established HIV infection with viral load >1000 copies/mL in the 4 weeks before delivery (if viral load is available)  
☒ Born to women with incident HIV infection during pregnancy or breastfeeding  
☐ Born to women identified for the first time during the postpartum period, with or without a negative HIV test prenatally  
☐ Other (please specify): \_\_\_\_\_

108. Does your country have a national recommendation on infant and young child feeding for HIV-exposed infants?  
☐ Yes, breastfeeding  
☒ Yes, replacement feeding  
☐ Yes, both are recommended, left to individual choice or different settings  
☐ No

108.1 If breastfeeding is recommended for HIV-positive women and HIV-exposed infants, is the recommended duration specified?  
☐ Yes (please specify the duration in months): \_\_\_\_\_  
☐ No

109. Is food and nutrition support in your country integrated within prevention of vertical transmission programmes?  
☐ Implemented in few (<50%) maternal and child health sites  
☐ Implemented in many (50–95%) maternal and child health sites  
☒ Implemented countrywide (>95% of maternal and child health sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

110. Does your country have a national strategy on interventions at delivery for women living with HIV who have not previously been tested for HIV?  
☒ Yes, fully implemented  
☐ Yes, partially implemented  
☐ Yes, but not implemented  
☐ No

111. Does your country have a policy on viral load testing for women during pregnancy, delivery and/or the post-partum/ breastfeeding period?  
☒ Yes  
☐ No

111.1 If yes, when is viral load testing done?

111.1.a During pregnancy  
☒ Yes  
☐ No

111.1.a.i If yes, which month of pregnancy: Monthly until suppressed

111.1.b At delivery  
☐ Yes  
☒ No

111.1.c Post-partum/breastfeeding  
☐ Yes  
☒ No

111.1.c.i If yes, how long after delivery (in months): \_\_\_\_\_

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#### Elimination of vertical transmission of syphilis

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**112. Does your country have a national plan for the elimination of vertical transmission of syphilis?**

- ☒ Yes, integrated with HIV or other elimination initiative(s)  
☐ Yes, stand-alone (not integrated with HIV or other elimination initiatives)  
☐ No national plan

**112.1 If yes, when was the national plan last updated?**

- ☐ 2018 or before  
☐ 2019  
☐ 2020  
☒ 2021  
☐ 2022  
☐ 2023

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**113. Does your country have a national policy for routinely screening pregnant women for syphilis?**

- ☒ Yes  
☐ No

**113.1 If yes, what tests are used:**

- ☐ Laboratory-based non-treponemal  
☐ Laboratory-based treponemal  
☒ Rapid syphilis treponemal tests  
☐ Dual HIV/syphilis rapid tests

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**114. Does your country have national guidelines to treat pregnant women with syphilis?**

- ☒ Yes  
☐ No

**114.1 If yes, is benzathine penicillin the first line of treatment recommended in national guidelines?**

- ☒ Yes  
☐ No

**114.2 If yes, does your policy allow the treatment of pregnant women with syphilis using benzathine penicillin in primary care facilities, including antenatal clinics, by any of the following (select all that apply)?**

- ☒ Nurses  
☒ Doctors  
☐ Other health care workers  
☐ None of the above

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**115. Does your country have a national policy on the clinical follow-up of infants born to syphilis-positive mothers??**

- ☒ Yes  
☐ No

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**116. Does the national definition for congenital syphilis include stillbirths?**

- ☒ Yes  
☐ No
-

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#### Prevention of vertical transmission of hepatitis B virus HBV

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117. Does your country have a national plan for the elimination of vertical transmission of HBV?

- ☐ Yes  
☒ No

118. Does your country have a policy on testing women for HBV during pregnancy?

- ☐ Yes  
☒ No

118.1 If yes, what is the status of implementation?

- ☐ Implemented in few (<50%) antenatal care clinics  
☐ Implemented in many (50–95%) antenatal care clinics  
☐ Implemented countrywide (>95%) of antenatal care clinics  
☐ Not implemented in practice  
☐ Other (please specify):

118.1.b If yes, do all HBsAg positive pregnant women have access to HBeAg or HBV DNA testing?

- ☐ Yes  
☐ No

119. Does your country have a national recommendation on hepatitis B testing among infants exposed to HBV?

- ☐ Yes  
☒ No

119. If yes, please specify the age (in months) at which infants exposed to HBV are tested for HBV: \_\_\_\_\_

120. Does your country have a policy on universal timely hepatitis B birth dose vaccination to all newborns (provided within 24 hours of birth)?

- ☐ Yes  
☒ No

120.1 If no, does your country have a policy on targeted timely hepatitis B birth dose vaccination to all newborns exposed to HBV (provided within 24 hours of birth)?

- ☐ Yes  
☒ No

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#### Infant diagnosis

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121. Do your national guidelines recommend that HIV-exposed infants be tested for HIV as follows (select all that apply)?

- ☐ Nucleic acid testing at birth  
☒ Nucleic acid testing at 6 weeks  
☐ Nucleic acid testing at 9 months  
☒ Antibody test at 18 months  
☒ Antibody test after 3 months from cessation of breastfeeding  
☒ Other (please specify): Nucleic Acid testing at 2 weeks if HIGH RISK and Nucleic Acid testing at 4 months

122. In addition to prevention of vertical transmission settings, do any of the following sites in your country carry out HIV testing of children (select all that apply)?

- ☐ Paediatric inpatient wards  
☐ Nutrition centres  
☐ Immunization clinics  
☐ Outpatient clinics  
☐ Tuberculosis (TB) clinics  
☒ Other (please specify): Sample collection occurs at paediatric inpatient wards, nutrition centres and outpatient clinics but testing for children is centralized
-

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**123. Does your country have a policy or recommendation for point-of-care infant diagnosis testing?**

- ☐ Yes  
☒ No

**123.1 If yes, where is it implemented?**

- ☐ Implemented in few (<50%) sites  
☐ Implemented in many (50–95%) sites  
☐ Implemented countrywide (>95% of sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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**Child antiretroviral therapy**

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**124. Does your country have a national policy on the frequency of clinic visits for children who are established\* on antiretroviral therapy?**

- ☒ Yes  
☐ No

**124.1 If yes, please specify the frequency of clinic visits in the national policy:**

- ☐ Once a month  
☐ Every 2 months  
☒ Every 3 months  
☐ Every 6 months  
☐ Every 12 months  
☐ Other (please specify): \_\_\_\_\_

**124.2 If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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**125. Does your country have a national policy on how frequently children who are established\* on antiretroviral therapy should pick up antiretroviral medicine?**

- ☒ Yes  
☐ No

**125.1 If yes, please specify the frequency of antiretroviral medicine pick-up included in the national policy:**

- ☐ Once a month  
☐ Every 2 months  
☒ Every 3 months  
☐ Every 6 months  
☐ Every 12 months  
☐ Other (please specify): \_\_\_\_\_

**125.2 If yes, what is the status of implementation?**

- ☐ Implemented in few (<50%) treatment sites  
☐ Implemented in many (50–95%) treatment sites  
☒ Implemented countrywide (>95% of treatment sites)  
☐ Not implemented in practice  
☐ Other (please specify): \_\_\_\_\_

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**126. Are cohorts of children receiving antiretroviral therapy monitored (i.e., ensuring that these children are alive and receiving antiretroviral therapy) in national registers at six-month and 12-month intervals?**

- ☒ Yes  
☐ No
-

- 
127. **When is a child who initiated antiretroviral therapy considered lost to follow-up in your country?**
- ☐ Has not been seen for HIV care or pharmacy pick-up in 1 month
  - ☐ Has not been seen for HIV care or pharmacy pick-up in 2 months
  - ☒ Has not been seen for HIV care or pharmacy pick-up in 3 months
  - ☐ Has not been seen for HIV care or pharmacy pick-up in 6 months
- 
128. **Does your country have a strategy or plan to ensure that adolescents born with HIV are not lost to follow-up as they transition into adult HIV care?**
- ☒ Yes
  - ☐ No
- 
129. **Are growth monitoring and nutrition programmes for children integrated with HIV testing and treatment in your country?**
- ☐ Implemented in few (<50%) treatment sites
  - ☐ Implemented in many (50–95%) treatment sites
  - ☒ Implemented countrywide (>95% of treatment sites)
  - ☐ Not implemented in practice
  - ☐ Other (please specify): \_\_\_\_\_
-

#### 4. Gender equality and empowerment of women and girls

- Reduce to no more than 10% the number of women, girls and people living with, at risk of and affected by HIV who experience gender-based inequalities and sexual and gender-based violence.
- Ensure that 95% of women and girls of reproductive age have their HIV and sexual and reproductive health-care service needs met, including antenatal and maternal care, information and counselling

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##### Violence

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**130. Does your country have a national gender equality plan or strategy?**

- ☒ Yes  
☐ No

**130.1 If yes, is there a national mechanism with a mandate to monitor and review implementation?**

- ☒ Yes  
☐ No

**130.2 If yes, identify the national mechanism that has a mandate to monitor and review implementation (select all that apply)?**

- ☒ Parliamentary  
☒ Interministerial  
☒ Includes participation of women's groups  
☐ Includes participation of violence against women groups

**130.3 If yes, do government entities provide budgetary commitments for the implementation of the gender equality plan or strategy?**

- ☒ Yes  
☐ No

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**131. Does your country have a national plan or strategy to address gender-based violence\* and violence against women that includes HIV?**

- ☒ Yes  
☐ No

**131.1 If yes, is there a national mechanism with a mandate to monitor and review implementation?**

- ☒ Yes  
☐ No

**131.2 If yes, identify the national mechanism that has a mandate to monitor and review implementation (select all that apply)?**

- ☒ Parliamentary  
☒ Interministerial  
☒ Includes participation of women's groups  
☐ Includes participation of violence against women groups

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**132. If legislation on domestic violence exists, is it (select those that apply):**

- ☐ Specific to violence against women and girls  
☒ Inclusive of all forms of familiar and intimate partner violence  
☐ Country does not have legislation on domestic violence

**132.1 If legislation includes explicit criminalization of marital rape, are laws based on lack of consent, without requiring proof of physical force, coercion or resistance?**

- ☐ Yes, based on lack of consent  
☐ No, require evidence of force, coercion or resistance  
☒ Legislation does not explicitly criminalize marital rape

**132.2 If legislation on domestic violence exists, have there been any successful criminal prosecutions for domestic violence in the past two years?**

- ☐ Yes  
☐ No
-

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**133. Do government entities provide budgetary commitments for the implementation of legislation and national plans addressing violence against women through any of the following?**

**133.a Legally-mandated annual appropriations**

☒ Yes

☐ No

**133.b Annual sectoral allocations (e.g. health, education, justice)**

☒ Yes

☐ No

**133.c Allocating funding and/or incentives to women's or other nongovernmental organizations for relevant activities**

☒ Yes

☐ No

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**134. Does legislation include any of the following provisions related to violence against women in your country (select all that apply)?**

☒ Court injunctions for the safety and security of survivors

☐ Special prosecution units in law enforcement

☒ Free legal services or assistance

☒ Access to shelters or other safe housing

☐ Financial support to survivors

☐ Specialized police staff or unit to deal with violence against women

☐ Services for the person perpetrating violence

☐ Other (please specify): \_\_\_\_\_

**134.1 If any of the above provisions are included in legislation, are they inclusive of trans women?**

☐ Yes

☒ No

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**135. Are there national protocols for:**

**135.a Health professionals dealing with cases of violence against women?**

☒ Yes

☐ No

**135.b Law enforcement personnel dealing with cases of violence against women?**

☒ Yes

☐ No

**135.c Education personnel dealing with cases of sexual abuse against children and other forms of gender-based violence\* against girls in school environments (select all that apply)?**

☒ Yes, for violence against girls

☒ Yes, for violence against boys

☐ Yes, for violence against gender-diverse children

☐ No

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**136. Does your country have any of the following to protect people living with HIV from violence (select all that apply)?**

☒ General criminal laws prohibiting violence

☐ Specific legal provisions prohibiting violence against people based on their HIV status

☒ Programmes to address intimate partner violence\*

☒ Programmes to address workplace violence

☒ Interventions to address police abuse

☒ Interventions to address torture and ill treatment in prisons and other closed settings

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**137. Does your country have any of the following to protect key populations from violence (select all that apply)?**

☒ General criminal laws prohibiting violence

☐ Specific legal provisions prohibiting violence against people based on belonging to a key population

☒ Programmes to address intimate partner violence\*

☐ Programmes to address workplace violence

☒ Interventions to address police abuse

☒ Interventions to address torture and ill treatment in prisons and other closed settings

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138. Does your country have at least one service delivery point that provides elements of comprehensive post-rape care as per World Health Organization (WHO) guidelines? The elements are: (1) first-line support, psychological first aid and psychosocial support; (2) emergency contraception; (3) sexually transmitted infection prophylaxis or treatment; (4) HIV post-exposure prophylaxis (PEP); and (5) safe abortion to the full extent of the law.

☐ Yes, provides all 5 elements

☒ Yes, provides 4 out of 5 elements

☐ Yes, provides 1 to 3 elements

☐ No services delivery point provides any of these elements

138.1 If yes, what proportion of health facilities provides each of the following elements of comprehensive post-rape care as per World Health Organization (WHO) guidelines:

138.1.a First-line support, psychological first aid and psychosocial support

☐ <50% of health facilities

☐ 50–80% of health facilities

☒ >80% of health facilities

☐ Not provided in any health facility

☐ Don't know

138.1.b Emergency contraception

☐ <50% of health facilities

☐ 50–80% of health facilities

☒ >80% of health facilities

☐ Not provided in any health facility

☐ Don't know

138.1.c Sexually transmitted infection treatment or prophylaxis

☐ <50% of health facilities

☐ 50–80% of health facilities

☒ >80% of health facilities

☐ Not provided in any health facility

☐ Don't know

138.1.d HIV post-exposure prophylaxis (PEP)

☐ <50% of health facilities

☐ 50–80% of health facilities

☒ >80% of health facilities

☐ Not provided in any health facility

☐ Don't know

138.1.e Safe abortion to the full extent of the law

☐ <50% of health facilities

☐ 50–80% of health facilities

☐ >80% of health facilities

☒ Not provided in any health facility

☐ Don't know

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## 5. Community leadership

- Ensure that community-led organizations deliver 30% of testing and treatment services by 2025, with a focus on HIV testing, linkage to treatment, adherence and retention support, and treatment literacy.
- Ensure that community-led organizations deliver 80% of HIV prevention services for populations at high risk of HIV infection by 2025, including for women within those populations.
- Ensure that community-led organizations deliver 60% of programmes to support the achievement of societal enablers by 2025.

### 139. Are there any laws, regulations or policies that provide for the registration of community-led organizations in your country (select all that apply)?

- ☒ Registration of organizations led by people living with HIV is possible
- ☒ Registration of organizations led by sex workers is possible
- ☒ Registration of organizations led by gay men and other men who have sex with men is possible
- ☒ Registration of organizations led by transgender people is possible
- ☐ Registration of organizations led by people who inject drugs is possible
- ☐ There are no laws, regulations or policies that provide for the registration of community-led organizations in the country
- ☐ Other (please specify): \_\_\_\_\_

### 140. Are there laws, policies or regulations that enable access to funding for community-led organizations (select all that apply)?

- ☐ Social contracting allowing for funding of service delivery by communities from domestic funding
- ☒ From international donors
- ☐ Require a certain percentage of government funding for community-led organizations to be allowed to operate
- ☒ No laws enabling access to funding, but community-led organizations are able to access funding under general laws, policies or regulations
- ☐ There are no laws, policies or regulations enabling access to funding for community-led organizations
- ☐ Other (please specify): \_\_\_\_\_

### 141. Are representatives of community-led organizations included in the national HIV coordinating mechanism or equivalent?

- ☐ There is no national HIV coordinating mechanism or equivalent
- ☐ There is a national HIV coordinating mechanism or equivalent, but it does not include any representatives of community-led organizations
- ☒ There is a national HIV coordinating mechanism or equivalent, and it includes representatives of community-led organizations

### 142. Can community-led organizations legally provide any of the following services (select all that apply)?

- ☒ Linkage to HIV treatment
- ☒ Adherence and retention support
- ☒ Treatment literacy
- ☐ Distribution of antiretroviral medicines
- ☒ Distribution of condoms and lubricants
- ☒ HIV testing
- ☐ Needle-syringe distribution
- ☐ Naloxone distribution
- ☒ Legal literacy
- ☒ Legal services
- ☒ Information on life skills-based HIV and sexuality education\*

## Participation

### 143. Do people living with HIV participate\* in developing national policies, guidelines and/or strategies relating to their health in your country?

- ☒ Yes
- ☐ No

### 144. Do women living with HIV participate\* in developing national policies, guidelines and strategies relating to prevention of vertical transmission?

- ☒ Yes
- ☐ No

### 145. Do gay men and other men who have sex with men participate\* in developing national policies, guidelines and/or strategies relating to their health in your country?

- ☒ Yes
- ☐ No

### 146. Do sex workers participate\* in developing national policies, guidelines and strategies relating to their health in your country?

- ☒ Yes
- ☐ No

|                                     |                                                                                                                                                                                    |
|-------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 147.                                | Do people who inject drugs participate* in developing national policies, guidelines and strategies relating to their health in your country?                                       |
| <input type="checkbox"/>            | Yes                                                                                                                                                                                |
| <input checked="" type="checkbox"/> | No                                                                                                                                                                                 |
| 148.                                | Do transgender people participate* in developing national policies, guidelines and strategies relating to their health in your country?                                            |
| <input checked="" type="checkbox"/> | Yes                                                                                                                                                                                |
| <input type="checkbox"/>            | No                                                                                                                                                                                 |
| 149.                                | Do former or current people in prisons and other closed settings participate* in developing national policies, guidelines and strategies relating to their health in your country? |
| <input type="checkbox"/>            | Yes                                                                                                                                                                                |
| <input checked="" type="checkbox"/> | No                                                                                                                                                                                 |
| 150.                                | Do young people (aged 15–24 years) participate* in developing national policies, guidelines and strategies relating to their health in your country?                               |
| <input checked="" type="checkbox"/> | Yes                                                                                                                                                                                |
| <input type="checkbox"/>            | No                                                                                                                                                                                 |
| 150.1                               | If yes, do young people participate* in any of the following decision-making spaces in the national HIV response (where they exist)?                                               |

| Decision-making space                                                                                                                                 | Does it exist? | Do young people participate in this space? |
|-------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|--------------------------------------------|
| Technical teams for the development, review and update of national AIDS strategies and plans                                                          | YES            | YES                                        |
| Technical teams for the development or review of programmes that relate to young people's access to HIV testing, treatment, care and support services | YES            | YES                                        |
| National AIDS Coordinating Authority or equivalent, with a broad-based multisector mandate                                                            | NO             | NO                                         |
| Global Fund to Fight AIDS, Tuberculosis and Malaria (Global Fund) Country Coordinating Mechanism                                                      | YES            | YES                                        |
| Community advisory body for hospitals, clinics and/or research projects                                                                               | NO             | NO                                         |
| Other (please specify): _____                                                                                                                         | Yes/No         | Yes/No                                     |

## 6. Realize human rights and eliminate stigma and discrimination

- Ensure that less than 10% of countries have restrictive legal and policy frameworks that unfairly target people living with, at risk of and affected by HIV by 2025. Examples include age of consent laws and laws related to HIV nondisclosure, exposure and transmission, laws that impose HIV-related travel restrictions and mandatory testing, and laws that lead to the denial or limitation of access to services by 2025.
- Invest US\$ 3.1 billion in societal enablers—including protection of human rights, reduction of stigma and discrimination and law reform, where appropriate—in low- and middle-income countries by 2025.
- Ensure that less than 10% of people living with, at risk of and affected by HIV experience stigma and discrimination by 2025.

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### Punitive laws

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**151. Does your country have laws at national or sub-national level criminalizing HIV nondisclosure, exposure or transmission?**

- ☐ Yes, HIV nondisclosure, exposure or transmission is a specific crime
- ☐ Yes, HIV nondisclosure, exposure or transmission is a crime under a law that covers a broader range of communicable diseases and mentions HIV
- ☐ Yes, HIV nondisclosure, exposure or transmission is a crime under a law that covers a broader range of communicable disease but does not specifically mention HIV
- ☒ No, but the general law has been used to prosecute cases in the past 10 years
- ☐ No

**151.1 If HIV non-disclosure, exposure or transmission are criminalized, either under a specific law or general law, is it limited to cases where the individual knows they have HIV, intends to transmit HIV and actually transmits it as per the 2013 recommendations on Ending overly broad criminalization of HIV non-disclosure, exposure and transmission: Critical scientific, medical and legal considerations<sup>9</sup>?**

- ☐ Yes, the criminal law is written to restrict it to these cases.
- ☐ No, but prosecutorial guidelines restrict it to these cases.
- ☐ No

**151.2 If HIV non-disclosure, exposure or transmission are criminalized, are there parts of the country where HIV nondisclosure, exposure or transmission are not criminalized?**

- ☐ Yes
- ☐ No

**151.3 If HIV nondisclosure, exposure or transmission are criminalized, has there been a moratorium on these laws in the last two years?**

- ☐ Yes
- ☐ No

**151.4 If HIV nondisclosure, exposure or transmission are criminalized, have any legal actions to decriminalize HIV nondisclosure, exposure or transmission either started or been in process in the last two years (select all that apply)?**

- ☐ Strategic litigation
- ☐ Proposal put before parliament
- ☐ Other (please specify): \_\_\_\_\_

**151.5 If HIV nondisclosure, exposure or transmission are criminalized, does that include vertical transmission?**

- ☐ Yes
- ☐ No

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**152. Is HIV mentioned as an aggravating factor in the law?**

- ☐ Yes
- ☒ No
- 

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<sup>9</sup> Ending Overly Broad Criminalization Of Hiv Non-Disclosure, Exposure And Transmission: Critical Scientific, Medical And Legal Considerations. Geneva: Unaid; 2013 ([https://www.unaids.org/Sites/Default/Files/Media\\_asset/20130530\\_guidance\\_ending\\_criminalisation\\_0.Pdf](https://www.unaids.org/Sites/Default/Files/Media_asset/20130530_guidance_ending_criminalisation_0.Pdf)).

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153. Are there any laws at national or sub-national level that criminalize transgender people (e.g. for impersonating the opposite sex or cross-dressing)?

☐ Yes

☒ No

153.1 If yes, are there any parts of the country where transgender people are not criminalized?

☐ Yes

☒ No

153.2 If transgender people are criminalized, has there been a moratorium on these laws in the last two years?

☐ Yes

☒ No

153.3 If transgender people are criminalized, have any legal actions to decriminalize transgender people either started or been in process in the last two years (select all that apply)?

☐ Strategic litigation

☐ Proposal put before parliament

☒ Other (please specify): No

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154. Is sex work criminalized or otherwise subject to punitive regulations in your country at national or sub-national level (select all that apply)?

☒ Yes, selling sexual services is criminalized

☒ Yes, buying sexual services is criminalized

☒ Yes, ancillary activities associated with selling sexual services are criminalized

☒ Yes, ancillary activities associated with buying sexual services are criminalized

☒ Yes, profiting from organizing and/or managing sexual services is criminalized

☒ Yes, other punitive regulation of sex work

☐ Sex work is not subject to punitive regulations and is not criminalized anywhere in the country

154.1 If yes to question 150, are there parts of the country where sex work is not criminalized or subject to punitive regulations?

☐ Yes

☒ No

154.2 If there are laws criminalizing any aspect of sex work, has there been a moratorium in the last two years?

☐ Yes

☒ No

154.3 If sex work is criminalized, have any legal actions to decriminalize sex work either started or been in process in the last two years (select all that apply)?

☐ Strategic litigation

☐ Proposal put before parliament

☒ Other (please specify): No

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155. Does your country have laws at national or subnational level criminalizing consensual same-sex sexual acts in private?

☐ Yes, death penalty

☒ Yes, imprisonment (14 years–life)

☒ Yes, imprisonment (up to 14 years)

☐ Yes, penalty not specified

☐ No, laws penalizing consensual same-sex sexual acts have been decriminalized or have never existed anywhere in the country

155.1 If yes, are there parts of the country where consensual same-sex sexual acts in private are not criminalized?

☐ Yes

☒ No

155.2 If there are laws criminalizing same-sex sexual acts, has there been a moratorium in the last two years?

☐ Yes

☒ No

155.3 If same-sex sexual acts are criminalized, have any legal actions to decriminalize same-sex sexual acts either started or been in process in the last two years (select all that apply)?

☐ Strategic litigation

☐ Proposal put before parliament

☒ Other (please specify): No

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**156. Are drug use and/or possession for personal use an offence in your country at the national or sub-national level?**

☐ Yes, a criminal offence for all drugs

☒ Yes, a criminal offence for all drugs except marijuana

☐ Yes, an administrative/non-criminal offence for all drugs

☐ No, not an offence anywhere in the country

**156.1 If yes, are there parts of the country where they are not criminal offences?**

☐ Yes

☒ No

**156.2 If drug use and/or possession for personal use are criminal offences, has there been a moratorium in the last two years?**

☐ Yes

☒ No

**156.3 If drug use and/or possession for personal use are criminal offences, have any legal actions to decriminalize drug use or possession for personal use either started or been in process in the last two years (select all that apply)?**

☐ Strategic litigation

☐ Proposal put before parliament

☒ Other (please specify): No

**156.4 If drug use and/or possession are administrative or non-criminal offences, are any of the following applied in your country for people who use drugs (select all that apply)?**

☐ Compulsory detention or compulsory rehabilitation in a closed facility

☐ Compulsory detention or rehabilitation outside of a facility

☐ Restrictions on movement, including suspension of passport or driving licence

☐ Mandatory counselling

☐ Fines

☒ Other administrative penalties (please specify): Marijuana is ticketable and not punitive

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**157. Does the law use quantity thresholds to estimate when drugs are possessed for personal use?**

☐ Yes, for all drugs

☒ Yes, only for marijuana

☐ No

**157.1 If yes, does the law specify the amount of drugs considered to be for personal use?**

☒ Yes

☐ No

**157.2 If yes, is this amount:**

☐ Indicative

☒ Determinative

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**158. Does your country apply compulsory detention or compulsory rehabilitation in a closed facility for people who use drugs?**

☐ Yes

☒ No

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**159. Apart from criminalization, does your country have other punitive laws affecting key populations (transgender people, men who have sex with men, sex workers and people who use drugs) (select all that apply)?**

☐ Yes, promotion ("propaganda") laws

☒ Yes, morality laws or religious laws that limit lesbian, gay, bisexual, transgender and intersex freedom of expression and association

☒ Yes, vagrancy laws

☐ Yes, others (please specify): \_\_\_\_\_

☐ No

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## Legal protections

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**160. Does your country have laws protecting against discrimination on the basis of HIV status?**

- ☐ Yes, constitutional or legislative protections that specify HIV status as a protected attribute
- ☐ Yes, constitutional or legislative protections that protect against discrimination on the basis of HIV status under another status (such as health, disability or "other status")
- ☒ No

**160.1 If yes, do constitutional or legislative protections apply to any of the following (select all that apply)?**

- ☐ Employment
- ☐ Education
- ☐ Health care
- ☐ Provision of other goods and services
- ☐ Other (please specify): \_\_\_\_\_

**161. Does your country have constitutional or legislative protections against discrimination on the basis of gender identity?**

- ☐ Yes, gender identity is specified as a protected attribute
- ☐ Yes, courts or government have legally recognized that gender identity is protected under another attribute (e.g. gender, sex, other status) (please specify): \_\_\_\_\_
- ☐ Yes, other (please specify): \_\_\_\_\_
- ☒ No

**162. Does your country have legal gender recognition laws or policies that enable the legal change of gender?**

- ☐ Yes
- ☒ No

**162.1 If yes, are any of the following required in order to change gender (select all that apply)?**

- ☐ Self-declaration (no medical evidence required)
- ☐ Gender reassignment surgery
- ☐ Sterilization
- ☐ Certificate of psychological assessment
- ☐ Other (please specify): \_\_\_\_\_

**162.2 If yes, is it legally possible to change one's gender marker on the following (select all that apply)?**

- ☐ Passport or other national ID
- ☐ Own birth certificate
- ☐ Marriage/divorce certificate

**163. Is it legally possible to change one's name to that of another gender on any of the following (select all that apply)?**

- ☐ Passport or other national ID
- ☐ Own birth certificate
- ☐ Marriage/divorce certificate
- ☒ No

**164. Does your country have constitutional or legislative protections against discrimination based on the involvement in sex work (e.g. occupation or profession)?**

- ☐ Yes, courts or government have legally recognized that sex work is protected under another attribute (please specify): \_\_\_\_\_
- ☐ Yes, other (please specify): \_\_\_\_\_
- ☒ No

**165. Does your country have constitutional or legislative protections against discrimination on the basis of sexual orientation?**

- ☐ Yes, sexual orientation is specified as a protected attribute
- ☐ Yes, courts or government have legally recognized that sexual orientation is protected under another attribute (e.g. gender, sex, other status) (please specify): \_\_\_\_\_
- ☐ Yes, other (please specify): \_\_\_\_\_
- ☒ No

**166. Does your country have constitutional or legislative protections against discrimination based on the involvement in drug use or possession?**

- ☐ Yes, courts or government have legally recognized that drug use or possession is protected under another attribute (please specify): \_\_\_\_\_
- ☐ Yes, other (please specify): \_\_\_\_\_
- ☒ No
-

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167. Is there explicit supportive reference to harm reduction in national policies?

- ☐ Yes  
☒ No

167.1 If yes, do policies address the specific needs of women who inject drugs?

- ☐ Yes  
☐ No

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168. Is drug use and/or possession a legal basis for removing children from parental custody?

- ☒ Yes  
☐ No

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Parental/guardian and spousal consent for accessing services

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169. Does your country have laws and/or policies requiring parental/guardian consent for adolescents (under age 18) to access hormonal or long-lasting contraceptives?

- ☐ Yes, for adolescents aged 17–18 years  
☒ Yes, for adolescents aged 15–16 years  
☒ Yes, for adolescents aged 13–14 years  
☒ Yes, for adolescents aged 12 years or younger  
☐ No age specified—ability to consent is based on an assessment of maturity  
☐ Other (please specify): \_\_\_\_\_  
☐ Not addressed in law/policy

169.1 If yes, are there exceptions for adolescents below the age of legal consent to be able to access hormonal or long-lasting contraceptives without parental/guardian consent (select all that apply)?

- ☐ Yes, demonstrated maturity  
☐ Yes, emancipated minors  
☐ Yes, pregnant adolescents  
☐ Yes, other (please specify): \_\_\_\_\_  
☒ No

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170. Does your country have laws and/or policies requiring parental/guardian consent for adolescents (under age 18) to access HIV testing and receive the results?

- ☐ Yes, for adolescents aged 17–18 years  
☒ Yes, for adolescents aged 15–16 years  
☒ Yes, for adolescents aged 13–14 years  
☒ Yes, for adolescents aged 12 years or younger  
☐ No age specified—ability to consent is based on an assessment of maturity  
☐ Other (please specify): \_\_\_\_\_  
☐ Not addressed in law/policy

170.1 If yes, are there exceptions for adolescents below the age of legal consent to be able to access HIV testing and receive the results without parental/guardian consent (select all that apply)?

- ☐ Yes, demonstrated maturity  
☐ Yes, emancipated minors  
☐ Yes, pregnant adolescents  
☐ Yes, other (please specify): \_\_\_\_\_  
☒ No
-

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**171. Does your country have laws and/or policies requiring parental/guardian consent for adolescents (under age 18) to access HIV self-testing and receive the results?**

☐ Yes, for adolescents aged 17–18 years

☒ Yes, for adolescents aged 15–16 years

☒ Yes, for adolescents aged 13–14 years

☒ Yes, for adolescents aged 12 years or younger

☐ No age specified—ability to consent is based on an assessment of maturity

☐ Other (please specify): \_\_\_\_\_

☐ Not addressed in law/policy

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**171.1 If yes, are there exceptions for adolescents below the age of legal consent to be able to access HIV self-testing and receive the results without parental/guardian consent (select all that apply)?**

☐ Yes, demonstrated maturity

☐ Yes, emancipated minors

☐ Yes, pregnant adolescents

☐ Yes, other (please specify): \_\_\_\_\_

☒ No

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**172. Does your country have laws and/or policies requiring parental/guardian consent for adolescents (under age 18) to access HIV treatment?**

☐ Yes, for adolescents aged 17–18 years

☒ Yes, for adolescents aged 15–16 years

☒ Yes, for adolescents aged 13–14 years

☒ Yes, for adolescents aged 12 years or younger

☐ No age specified—ability to consent is based on an assessment of maturity

☐ Other (please specify): \_\_\_\_\_

☐ Not addressed in law/policy

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**172.1 If yes, are there exceptions for adolescents below the age of legal consent to be able to access HIV treatment without parental/guardian consent (select all that apply)?**

☐ Yes, demonstrated maturity

☐ Yes, emancipated minors

☐ Yes, pregnant adolescents

☐ Yes, other (please specify): \_\_\_\_\_

☒ No

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**173. Does the law allow adolescents (under age 18) to access the following services:**

**173.a Opioid agonist maintenance therapy**

☐ Yes

☐ No     n/a Opioid agonist maintenance therapy not a service offered

**173.a.i If yes, is parental/guardian consent required to access?**

☐ Yes

☐ No

**173.a.ii If yes, please specify until what age parental/guardian consent is required to access: \_\_\_\_\_**

**173.b Needle and syringe programmes**

☐ Yes

☐ No     n/a Needles and syringe programmes not a service offered

**173.b.i If yes, is parental/guardian consent required to access?**

☐ Yes

☐ No

**173.b.ii If yes, please specify until what age parental/guardian consent is required to access: \_\_\_\_\_**

**173.c Naloxone**

☐ Yes

☐ No     n/a Naloxone not a service offered

**173.c.i If yes, is parental/guardian consent required to access?**

☐ Yes

☐ No

**173.c.ii If yes, please specify until what age parental/guardian consent is required to access: \_\_\_\_\_**

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174. Does your country have laws or policies requiring parental or guardian consent for adolescents aged under 18 years to access pre-exposure prophylaxis (PrEP)?

- ☐ Yes, for adolescents aged 17–18 years  
☒ Yes, for adolescents aged 15–16 years  
☒ Yes, for adolescents aged 13–14 years  
☒ Yes, for adolescents aged 12 years or younger  
☐ No age specified—ability to consent is based on an assessment of maturity  
☐ Other (please specify): \_\_\_\_\_  
☐ Not addressed in law/policy

174.1 If yes, are there exceptions for adolescents under the age of legal consent to access HIV treatment without parental or guardian consent (select all that apply)?

- ☐ Yes, demonstrated maturity  
☐ Yes, emancipated minors  
☐ Yes, pregnant adolescents  
☐ Yes, other (please specify): \_\_\_\_\_  
☒ No

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175. Does your country have laws requiring spousal consent for married women to access any sexual or reproductive health services?

- ☒ Yes **Consent required for sterilization**  
☐ No

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176. Does your country have laws requiring spousal consent for married women to access HIV testing?

- ☐ Yes  
☒ No
-

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## Stigma and discrimination

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**177. Does your country have training programmes for the following groups on human rights and nondiscrimination legal frameworks as applicable to HIV?**

**177.a For police and other law enforcement personnel**

- ☒ Yes, as part of formative training curricula at the national level
- ☒ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)
- ☒ Yes, as a recurrent part of on-the-job training at the national level
- ☒ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
- ☒ Yes, as one-off or ad hoc activities
- ☐ No

**177.b For members of the judiciary**

- ☐ Yes, as part of formative training curricula at the national level
- ☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)
- ☐ Yes, as a recurrent part of on-the-job training at the national level
- ☒ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
- ☒ Yes, as one-off or ad hoc activities
- ☐ No

**177.c For elected officials (lawmakers/parliamentarians)**

- ☐ Yes, as a recurrent part of on-the-job training at the national level
- ☐ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
- ☒ Yes, as one-off or ad hoc activities
- ☐ No

**177.d For health-care workers**

- ☒ Yes, as part of formative training curricula at the national level
- ☒ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)
- ☒ Yes, as a recurrent part of on-the-job training at the national level
- ☒ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
- ☐ Yes, as one-off or ad hoc activities
- ☐ No

**177.e For educators**

- ☐ Yes, as part of formative training curricula at the national level
  - ☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)
  - ☒ Yes, as a recurrent part of on-the-job training at the national level
  - ☒ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
  - ☐ Yes, as one-off or ad hoc activities
  - ☐ No
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**178.** In the past two years, has the government provided and/or funded training for the following groups on human rights and nondiscrimination relating to key populations in at least 25% of provinces/regions/districts (select all that apply)?

**178.a For police and other law enforcement personnel**

- ☒ Sex workers
- ☒ Gay men and other men who have sex with men
- ☒ Transgender people
- ☐ People who inject drugs
- ☒ People in prisons and other closed settings

**178.b For members of the judiciary**

- ☒ Sex workers
- ☒ Gay men and other men who have sex with men
- ☒ Transgender people
- ☐ People who inject drugs
- ☐ People in prisons and other closed settings

**178.c For elected officials (lawmakers/parliamentarians)**

- ☒ Sex workers
- ☒ Gay men and other men who have sex with men
- ☒ Transgender people
- ☐ People who inject drugs
- ☐ People in prisons and other closed settings

**178.d For health-care workers**

- ☒ Sex workers
- ☒ Gay men and other men who have sex with men
- ☒ Transgender people
- ☐ People who inject drugs
- ☐ People in prisons and other closed settings

**178.e For educators**

- ☐ Sex workers
- ☐ Gay men and other men who have sex with men
- ☐ Transgender people
- ☐ People who inject drugs
- ☐ People in prisons and other closed settings

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**179.** Does your country have training programmes on the prevention of violence against women and gender-based violence for the following groups?

**179.a For police and other law enforcement personnel**

- ☒ Yes, as part of formative training curricula at the national level
  - ☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)
  - ☒ Yes, as a recurrent part of on-the-job training at the national level
  - ☒ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)
  - ☐ Yes, as one-off or ad hoc activities
  - ☐ No
-

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**179.b For members of the judiciary**

☐ Yes, as part of formative training curricula at the national level

☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)

☐ Yes, as a recurrent part of on-the-job training at the national level

☐ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)

☐ Yes, as one-off or ad hoc activities

☒ No

**179.c For elected officials (lawmakers/parliamentarians)**

☐ Yes, as part of formative training curricula at the national level

☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)

☐ Yes, as a recurrent part of on-the-job training at the national level

☐ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)

☐ Yes, as one-off or ad hoc activities

☒ No

**179.d For health-care workers**

☐ Yes, as part of formative training curricula at the national level

☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)

☐ Yes, as a recurrent part of on-the-job training at the national level

☐ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)

☒ Yes, as one-off or ad hoc activities

☐ No

**179.e For educators**

☐ Yes, as part of formative training curricula at the national level

☐ Yes, as part of formative training curricula at the subnational level (in at least one province/region/district)

☐ Yes, as a recurrent part of on-the-job training at the national level

☐ Yes, as a recurrent part of on-the-job training at the subnational level (in at least one province/region/district)

☒ Yes, as one-off or ad hoc activities

☐ No

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**180. Are there public accountability mechanisms established by the government through which people can make complaints and seek redress for cases of HIV-related discrimination (based on perceived HIV status and/or belonging to any key population)? Examples include (but are not limited to) a national human rights institution, ombudsperson, tribunal or commission.**

☒ Yes (please describe): MOHW Complaint Management System

☐ No

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**181. Does your country have any of the following human rights monitoring and enforcement mechanisms?**

**181.a Existence of independent functional national institutions for the promotion and protection of human rights, including human rights commissions, law reform commissions, watchdogs and ombudspersons that consider HIV-related issues within their work**

☒ Yes

☐ No

**181.b Oversight for implementation of concluding observations and recommendations from treaty monitoring bodies and Universal Periodic Review (UPR)**

☒ Yes

☐ No

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182.

**Does your country have mechanisms in place for accessing affordable legal services (select all that apply)?**

- ☒ Yes, publicly-funded legal aid systems applicable to HIV casework
  - ☒ Yes, pro bono legal services provided by private law firms
  - ☒ Yes, legal services provided by (university-based) legal clinics
  - ☒ Yes, community paralegals
  - ☐ Yes, other (please describe): \_\_\_\_\_
  - ☐ No
-

## 7. Universal health coverage and integration

- Invest in robust, resilient, equitable and publicly funded systems for health and social protection systems that provide 90% of people living with, at risk of and affected by HIV with people-centred and context-specific integrated services for: HIV and other communicable diseases; noncommunicable diseases; sexual and reproductive health care; gender-based violence; mental health; palliative care; treatment of alcohol dependence; drug use legal services; and other services they need for their overall health and well-being Ensure that by 2025, 45% of people living with, at risk of and affected by HIV and AIDS have access to social protection benefits.
- Ensure that 90% of people in humanitarian settings have access to integrated HIV services.
- Ensure the systematic engagement of HIV responses in pandemic response infrastructure and arrangements, leveraging national HIV strategic plans to guide key elements of pandemic preparedness planning and ensuring that 95% of people living with, at risk of and affected by HIV are protected against pandemics, including COVID-19.

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### Cervical cancer

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**183. Have the recommendations in the 2021 World Health Organization (WHO) guidelines for screening of cervical pre-cancer lesions for cervical cancer prevention among women living with HIV been adopted in your country's national guidelines?**



Yes



No

**183.a If yes, please indicate the WHO recommendations adopted (select all that apply):**



Age to start regular cervical cancer screening is 25 years



HPV-DNA Is used as the primary screening test as part of a screen and treat approach



VIA is used as a primary screening test as part of a screen and treat approach



Cytology Is used as a primary screening test as part of a screen and treat approach



Cytology Is used as a primary screening test as part of a screen, triage and treat approach



Screening interval recommended for cytology/VIA is every 3 years



Screening interval recommended for HPV-DNA is every 3-5 years



Other (please specify): \_\_\_\_\_

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### Sexually transmitted infections

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**184. Does your country have a national strategy or action plan for the prevention and control of sexually transmitted infections?**



Yes



No

**184.1 If yes, in what year were they last updated?**



Before 2020



2020



2021



2022



2023



2024

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**185. Does your country have national case management guidelines for sexually transmitted infections**



Yes



No

**185.1 If yes, in what year were they last updated?**



Before 2020



2020



2021



2022



2023



2024

---

**186.** Does your country have national treatment guidelines for *Neisseria gonorrhoeae*?

- ☒ Yes  
☐ No

**186.1** If yes, in what year were they last updated?

- ☐ Before 2020  
☐ 2020  
☐ 2021  
☒ 2022  
☐ 2023  
☐ 2024

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**187.** Is gonococcal antimicrobial resistance monitoring conducted in the country?

- ☐ Yes, annually  
☐ Yes, less than once a year  
☒ No
-

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## Social protection

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**188.** Does the country have an approved social protection\* strategy, policy or framework?

- ☒ Yes, and it is being implemented  
☐ Yes, but it is not being implemented  
☐ No

**188.1** If yes:

**188.1.a** Does it refer to HIV?

- ☒ Yes  
☐ No

**188.1.b** Does it recognize people living with HIV as key beneficiaries?

- ☐ Yes  
☒ No

**188.1.b.i.** If no, please describe any conditions under which people living with HIV can access social protection benefits: **Social Assessment**

**188.1.c** Does it recognize any key populations (sex workers, gay men and other men who have sex with men, people who inject drugs, transgender people or people in prisons and other closed settings) as key beneficiaries?

- ☐ Yes  
☒ No

**188.1.c.i.** If yes, which key populations are recognized as key beneficiaries (select all that apply)?

- ☐ Sex workers  
☐ Gay men and other men who have sex with men  
☐ Transgender people  
☐ People who inject drugs  
☐ People in prisons and other closed settings

**188.1.c.ii** If no, please describe any conditions under which key populations can access social protection benefits: **Social Assessment**

**188.1.d** Does it recognize adolescent girls and young women as key beneficiaries?

- ☐ Yes  
☒ No

**188.1.e** Does it recognize children affected by HIV as key beneficiaries?

- ☐ Yes  
☒ No

**188.1.f** Does it recognize families affected by HIV as key beneficiaries?

- ☐ Yes  
☒ No

**188.1.g** Does it address the issue of unpaid care work in the context of HIV?

- ☐ Yes  
☒ No

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**189.** Are representatives of the National AIDS Programme or equivalent included in any social protection\* coordination mechanism or platform?

- ☐ There is no social protection coordination mechanism or platform  
☐ There is a social protection coordination mechanism or platform, but it does not include any representatives of the National AIDS Programme or equivalent  
☒ There is a social protection coordination mechanism or platform, and it includes representatives of the National AIDS Programme or equivalent

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**190.** Are any cash transfer\* programmes for young women aged 15–24 years being implemented in the country?

- ☒ Yes  
☐ No
- 

## Trade-related Aspects of Intellectual Property Rights

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| 191. What public health-related Trade-related Aspects of Intellectual Property Rights (TRIPS) flexibilities* are incorporated in your country's national legislation on industrial and/or intellectual property, and which of them have been used to enhance access to HIV-related products and/or other health products? |                                                                                    |                                                               |                                                  |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------|---------------------------------------------------------------|--------------------------------------------------|
| Public health-related TRIPS flexibilities                                                                                                                                                                                                                                                                                 | Are they incorporated in national legislation on industrial/intellectual property? | If yes, what is the most recent year in which they were used? | If yes, for which commodity have they been used? |
| Exhaustion of rights (parallel importation) (Article 6)                                                                                                                                                                                                                                                                   | <b>Yes</b>                                                                         | Yes/No                                                        | Yes/No                                           |
| Patentable subject matter (Article 27)                                                                                                                                                                                                                                                                                    | <b>Yes</b>                                                                         |                                                               |                                                  |
| Research and experimental use exception (Article 30)                                                                                                                                                                                                                                                                      | <b>Yes</b>                                                                         | Yes/No                                                        | Yes/No                                           |
| Regulatory (Bolar) exception (Article 30)                                                                                                                                                                                                                                                                                 | <b>Yes</b>                                                                         | Yes/No                                                        | Yes/No                                           |
| Compulsory licensing (Article 31)                                                                                                                                                                                                                                                                                         | <b>Yes</b>                                                                         | Yes/No                                                        | Yes/No                                           |
| Public, noncommercial use (government use) (Article 31)                                                                                                                                                                                                                                                                   | <b>Yes</b>                                                                         | Yes/No                                                        | Yes/No                                           |
| Scope of pharmaceutical test data protection (Article 39.3)                                                                                                                                                                                                                                                               | <b>No</b>                                                                          | Yes/No                                                        | Yes/No                                           |
| Competition law (Article 40)                                                                                                                                                                                                                                                                                              | Yes/No                                                                             | Yes/No                                                        | Yes/No                                           |
| Transition periods (Articles 65.2, 65.4 and 66.1)                                                                                                                                                                                                                                                                         | <b>No</b>                                                                          | Yes/No                                                        | Yes/No                                           |

**Universal health insurance**

192. **Does your country have a universal health insurance scheme?**  
☒ Yes  
☐ No

192.1 **If no, is your country moving to a universal health insurance scheme?**  
☐ Yes  
☐ No

192.2 **If yes to questions 192 or 192.1, does the benefits package include the following?**

192.2.a **Antiretroviral medicines**  
☒ Yes  
☐ No

192.2.b **Pre-exposure prophylaxis (PrEP)**  
☒ Yes  
☐ No

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## Strategy

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**193. Does your country have a national strategy or policy that guides the AIDS response?**

- ☒ Yes, a stand-alone AIDS strategy or policy
- ☐ Yes, a health strategy or policy that integrates the AIDS response
- ☐ No
- ☐ Other (please specify): \_\_\_\_\_

**193.1 If yes, has the national HIV strategy or policy been reviewed in the past two years?**

- ☒ Yes
- ☐ No

**193.2 If yes to Q193, does the national strategy or policy guiding the AIDS response explicitly address the following key populations or vulnerable groups (select all that apply)?**

- ☒ Adolescent key populations
- ☒ Gay men and other men who have sex with men
- ☐ People in prisons and other closed settings
- ☐ People who inject drugs
- ☒ Sex workers (male and female)
- ☒ Transgender people
- ☐ Nondisplaced people affected by emergencies
- ☐ Refugees and asylum seekers
- ☐ Internally displaced people
- ☐ Migrants

**193.3 If yes to Q193, does the national strategy or policy guiding the AIDS response do the following (select all that apply)?**

- ☒ Specifically include explicit plans or activities that address the needs of key populations
- ☒ Specifically include explicit plans or activities that address the needs of young women and girls
- ☒ Specifically include explicit plans or activities that address HIV-related stigma and discrimination
- ☒ Specifically include explicit plans or activities relating to the reform of discriminatory or punitive laws
- ☒ Draw on the most recent evidence about the national HIV epidemic and the status of the response
- ☒ Integrate inputs from a multisectoral process, including various government sectors and nongovernmental partners
- ☐ Draw on evidence provided by community-led organizations

**193.4 If yes to Q193, does the national strategy or policy guiding the AIDS response include gender-transformative\* interventions, including interventions to address the intersections of gender-based violence\* and HIV?**

- ☒ Yes
- ☐ No

**193.4.a. If yes to Q193.4, does the national strategy or policy guiding the AIDS response include a dedicated budget for implementing gender-transformative\* interventions?**

- ☒ Yes
- ☐ No
-

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### Monitoring and evaluation

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- 194.** Does your country have a national monitoring and evaluation plan or strategy for HIV?
- ☒ Yes, a stand-alone HIV monitoring and evaluation strategy or plan
- ☐ Yes, HIV monitoring and evaluation is integrated in a broader health monitoring and evaluation strategy or plan
- ☐ No
- ☐ Other (please specify): \_\_\_\_\_
- 194.1** If yes, when was it last updated?
- ☐ 2019
- ☐ 2020
- ☐ 2021
- ☐ 2022
- ☒ 2023
- 194.2** If yes, does it integrate gender-sensitive indicators\* to help understand gender-based inequities and gender inequality?
- ☒ Yes
- ☐ No
- 194.3** Does your country have a data quality task team to review the subnational programme data on at least a semi-annual basis?
- ☒ Yes
- ☐ No
- 

### Information system

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- 195.** Are patient-level data routinely available within the health information system?
- ☐ Yes, fully electronic
- ☒ Yes, partially electronic
- ☐ Yes, paper-based only
- ☐ No health information system exists
- 195.1** If patient-level data exist, are antenatal care testing and treatment cascade data included in the health information system at the district level (select all that apply)?
- ☒ Pregnant women tested during antenatal care and outcome of test
- ☒ Women already on antiretroviral therapy at first antenatal care visit
- ☒ The number of people tested for HIV
- ☒ The number of people testing HIV-positive
- ☒ The number of people newly diagnosed with HIV who are on antiretroviral therapy
- ☒ The number of people on antiretroviral therapy who are virally suppressed
- Are data from community-led organizations on services they delivered integrated in the national health information system or equivalent?**
- ☐ Yes, community-led organizations are included as a type of provider for disaggregation of existing indicators
- ☐ Yes, specific indicators on service delivery by community-led organizations are included
- ☒ No
- ☐ Community-led organizations cannot provide services
-

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## Surveillance

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196. Is HIV a nationally notifiable condition by law?

- ☒ Yes  
☐ No
- 

197. Does the country have a national HIV case surveillance\* system?

- ☒ Yes  
☐ No

197.1 If yes, does the national HIV case surveillance system include the following (select all that apply)?

- ☒ Individual-level data for each person diagnosed with HIV  
☒ Collection of data from different sources (laboratories, testing and treatment records) to promote completeness of data on HIV case  
☒ Linkage of individual-level data to remove duplicate records  
☒ CD4 count at HIV diagnosis  
☒ Initiation of antiretroviral therapy  
☒ First and follow-up viral load test results  
☒ Pregnancy in women living with HIV  
☒ Death
-

## Patient monitoring systems

198. Has the country updated the HIV testing and treatment indicators and tools using the 2022 World Health Organization (WHO) Consolidated guidelines on person-centered HIV strategic information?

- ☒ Yes, fully  
☐ Yes, partially  
☐ No  
☐ Don't know

## Unique identification codes for patients

199. Does the country have a method to identify and remove duplicate health information for patients within and between clinics—such as linking records using unique identifiers and/or personal identifiable information (including biometrics)—for the following services?

|                                                                                                             | Method to identify and remove duplicate health information                                                                                                                                                                                                                               | If yes, please specify how data are linked                                                                                                                                                                                                                                                                                                                                       |
|-------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Treatment services</b>                                                                                   | <input checked="" type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know                            | <input checked="" type="checkbox"/> National unique personal identifier<br><input checked="" type="checkbox"/> HIV-specific unique identifier<br><input checked="" type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____                          |
| <b>Testing services</b>                                                                                     | <input type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input checked="" type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know                            | <input checked="" type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input checked="" type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____                                     |
| <b>Laboratory services</b>                                                                                  | <input checked="" type="checkbox"/> Yes, nationally harmonized <b>centralized testing</b><br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know | <input checked="" type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input checked="" type="checkbox"/> Other (please specify) <b>Medical Facility Unique Identifier</b> |
| <b>HIV prevention services designed for any key population group to track combination prevention uptake</b> |                                                                                                                                                                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                  |
| <b>Gay Men And Other Men Who Have Sex With Men</b>                                                          | <input checked="" type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know                            | <input checked="" type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input checked="" type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____                                     |
| <b>Sex workers</b>                                                                                          | <input checked="" type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know                            | <input checked="" type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input checked="" type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____                                     |
| <b>Transgender people</b>                                                                                   | <input checked="" type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know                            | <input checked="" type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input checked="" type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____                                     |

|                                |                                                                                                                                                                                                                                                               |                                                                                                                                                                                                                                                                                                                        |
|--------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>People who inject drugs</b> | <input type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input checked="" type="checkbox"/> No<br><input type="checkbox"/> Don't know | <input type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____ |
|--------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

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|                                         |                                                                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                        |
|-----------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Other (please specify):</b><br>_____ | <input type="checkbox"/> Yes, nationally harmonized<br><input type="checkbox"/> Yes, but varies across regions<br><input type="checkbox"/> Yes, but varies across programmes<br><input type="checkbox"/> No<br><input type="checkbox"/> Don't know | <input type="checkbox"/> National unique personal identifier<br><input type="checkbox"/> HIV-specific unique identifier<br><input type="checkbox"/> Combination of routinely collected personal identifying information<br><input type="checkbox"/> Biometric<br><input type="checkbox"/> Other (please specify) _____ |
|-----------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

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**199.1** ☒ **If yes to any of the above, does the unique identifier policy also provide for legally enforceable data privacy protections?**  
☐ Yes  
☐ No

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**95–95–95**

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**200.** **When was the most recent data quality review conducted to determine the accuracy of national-level numbers of people reported to be on treatment?**

☐ Currently being conducted (results expected next year)  
☒ Completed in the last year and results available  
☐ Completed 2 to 5 years ago  
☐ Never conducted or conducted more than 5 years ago

**200.1** **If a data quality review has been conducted in the last year, have the results been used to adjust the numbers of people on treatment reported in Indicator 2.2?**

☐ Yes  
☒ No

---

**201.** **When was the most recent data quality review conducted to determine the accuracy of the number of people reported to have suppressed viral loads?**

☐ Currently being conducted (results expected next year)  
☒ Completed in the last year and results available  
☐ Completed 2 to 5 years ago  
☐ Never conducted or conducted more than 5 years ago

**201.1** **If a data quality review has been conducted in the last year, has this been used to adjust the number of people who have suppressed viral loads reported in Indicator 2.3?**

☐ Yes  
☒ No

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**Data use**

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**202.** **Are data reviews of HIV treatment cascade data being conducted?**

☒ Yes  
☐ No

**202.1** **If yes, please select the frequency with which data reviews of HIV treatment cascade data are conducted.**

☒ Quarterly  
☐ Every 6 months  
☐ Annually  
☐ Other, please specify: \_\_\_\_\_

**202.2** **If yes, at which level are data reviews conducted (select all that apply)?**

☒ National  
☒ District  
☒ Facility

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## National Commitments and Policy Instrument: Part B

\* The guidelines for the NCPI define the terms marked with an asterisk (\*).

### 1. Combination prevention for all

- Reduce new HIV infections to under 370 000 by 2025. Ensure that 95% of people at risk of HIV infection, within all epidemiologically relevant groups, age groups and geographic settings, have access to and use appropriate, prioritized, person-centred and effective combination prevention options.
- Reduce the number of new HIV infections among adolescent girls and young women to below 50 000 by 2025.
- Ensure availability of pre-exposure prophylaxis (PrEP) for 10 million people at substantial risk of HIV and post-exposure prophylaxis (PEP) for people recently exposed to HIV by 2025.
- 95% of people within humanitarian settings at risk of HIV use appropriate, prioritized, people-centred and effective combination prevention options

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#### HIV prevention services for people who inject drugs

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1. Are needle-syringe programmes operational in your country?

☐

Yes

☐

No, not at all

☒

No, but needles and syringes can be legally purchased in pharmacies without a prescription

---

2. Is naloxone (used to reverse opioid overdoses) available through community distribution in your country?

☐

Yes

☒

No

---

3. Are opioid agonist maintenance therapy programmes operational in your country?

☐

Yes

☒

No

---

#### HIV prevention services in prisons and other closed settings

---

4. Are needle-syringe programmes operational in prisons and other closed settings in your country?

☐

Yes

☒

No

---

5. Are opioid agonist maintenance therapy programmes operational in prisons and other closed settings in your country?

☐

Yes

☒

No

---

6. Are condoms and lubricants available to people in prisons and other closed settings in your country?

☐

Yes

☒

No

---

7. Is antiretroviral therapy available to all people in prisons and other closed settings living with HIV in your country?

☒

Yes

☐

No

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---

**8.** Are HIV tests in prisons and other closed settings in your country:

**8.a** Carried out only with the informed consent of people in prisons and other closed settings?

☒ Yes

☐ No

**8.b** Systematically offered at entry and/or exit?

☒ Yes

☐ No

**8.c** Free of charge?

☒ Yes

☐ No

**8.d** Confidential?

☒ Yes

☐ No

**8.e** Available at any time during detention?

☒ Yes

☐ No

**8.f** Accompanied by relevant and accessible information?

☒ Yes

☐ No

**8.g** Accompanied by confidential pre- and post-test counselling?

☒ Yes

☐ No

**8.h** Equally accessible to all people in prisons and other closed settings?

☒ Yes

☐ No

**8.i** If no, which people in prisons and other closed settings do not have equal access (please specify): \_\_\_\_\_

---

**9.** Is hepatitis C treatment (using direct-acting antiviral medicines) available in prisons and other closed settings in your country?

☐ Yes

☒ No

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**10.** Is pre-exposure prophylaxis (PrEP) available through any of the following in your country (select all that apply)?

☒ Public health-care facilities

☒ Community-based distribution (including mobile services)

☐ Pharmacies (stand-alone, including online)

☒ Private health-care providers

☐ The Internet (informal purchases)

☐ Educational institutions (e.g. schools, universities)

☐ Research sites

☐ Other (please specify): \_\_\_\_\_

---

**11.** Do any of the following barriers limit access to pre-exposure prophylaxis (PrEP) in your country (select all that apply)?

☐ Possession of PrEP used as evidence of sex work or other criminalized sexual activity

☒ PrEP is associated with stigmatized sexual behaviour

☒ PrEP is only provided in specific geographic locations

☒ PrEP is only provided in specialized HIV treatment locations

☐ High out-of-pocket cost of PrEP services

☐ Eligibility criteria (e.g. PrEP is limited to certain populations)

☐ Negative attitudes of PrEP providers Inconvenient service provision (e.g. opening hours)

☐ PrEP is not available in the country

---

## 2. 95–95–95 for HIV testing and treatment

- Reduce annual AIDS-related deaths to under 250 000 by 2025.
  - Ensure that 34 million people are on HIV treatment by 2025.
  - Achieve the 95–95–95 testing, treatment and viral suppression targets within all demographics, groups and geographic settings, including children and adolescents living with HIV.
    - 95% of people living with HIV know their HIV status.
    - 95% of people who know their HIV-positive status are accessing treatment.
    - 95% of people on treatment have suppressed viral loads.
  - Ensure that 90% of people living with HIV receive preventive treatment for tuberculosis (TB) by 2025.
- Reduce TB-related deaths among people living with HIV by 80% by 2025 (compared to a 2010 baseline).

12.



**Does your country have any forms of mandatory (or compulsory) HIV testing that are provided for or carried out?**

No



Yes (please briefly explain when mandatory testing is carried out and the groups that are affected): \_\_\_\_\_

13.



**Where is viral load testing available in your country?**

Available at specialized centres only



Available at antiretroviral therapy facilities, either on-site or by referral



Other (please specify): \_\_\_\_\_

14.



**Are any of the following retention support services available in your country (select all that apply)?**

Community-based interventions (e.g. patient advocates, treatment and peer support interventions providing adherence and psychosocial support in the community)



Adherence clubs and peer support (e.g. peer support, distribution of antiretroviral medicines and assessment by nonclinical or lay providers)



Other (please specify): \_\_\_\_\_

15.



**Are any of the following adherence support services available in your country (select all that apply)?**



Peer counsellors



Text messages



Use of reminder devices



Patient reintegration follow-up (telephone calls/home visits)



Enhanced adherence counselling



Referral to psychological/socioeconomic support



Cognitive behavioural therapy



Behavioural skills training/medication adherence training



Fixed-dose combinations and once-daily regimens



Case management



Peer navigation



Other (please specify): \_\_\_\_\_

16.



**Are migrants<sup>1</sup> able to access HIV services (testing, antiretroviral medicines and care)?**



Yes



No

<sup>1</sup> Migrants are defined as non-citizens who are in a country other than their country of origin for a stay of longer than six months, excluding refugees and asylum seekers

### 3. End paediatric AIDS and eliminate vertical transmission

- Ensure that 75% of all children living with HIV have suppressed viral loads by 2023 and 86% by 2025, in line with the 95–95–95 HIV treatment targets.
- Ensure that 95% of pregnant women have access to testing for HIV, syphilis, hepatitis B and other sexually transmitted infections by 2025.
- Ensure that 95% of pregnant and breastfeeding women in high HIV burden settings have access to retesting during late pregnancy and in the post-partum period by 2025.
- Ensure that all pregnant and breastfeeding women living with HIV are receiving life-long antiretroviral therapy, with 95% achieving and sustaining viral suppression before delivery and during breastfeeding by 2025.
- Ensure that all HIV-negative pregnant and breastfeeding women in high HIV burden settings—or those who have male partners at high risk of HIV in all settings—have access to combination prevention, including pre-exposure prophylaxis (PREP), and that 90% of their male partners who are living with HIV are continuously receiving antiretroviral therapy.
- Ensure that 95% of HIV-exposed children are tested by two months of age and after the cessation of breastfeeding.

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**17. Do women living with HIV in your country participate\* in developing national policies, guidelines and strategies relating to the prevention of vertical HIV transmission?**

- ☒ Yes  
☐ No

---

**18. In the context of prevention of vertical HIV transmission programmes in your country, are there reports on (or documentation of) any of the following (select all that apply)?**

- ☐ Mandatory or coerced testing and/or treatment for HIV  
☐ Lack of informed, voluntary and prior obtained consent  
☐ Forced and coerced abortion, contraception and/or sterilization  
☒ Lack of confidentiality and privacy  
☒ Other grave or systematic human rights abuses\* (please describe): Intimate partner violence

**18.1 If there are reports of any of these situations in your country, is the government carrying out due diligence in responding to them?**

- ☐ Yes  
☒ No  
☐ Don't know
- 

### 4. Gender equality and empowerment of women and girls

- Reduce to no more than 10% the number of women, girls and people living with, at risk of and affected by HIV who experience gender-based inequalities and sexual and gender-based violence.
- Ensure that 95% of women and girls of reproductive age have their HIV and sexual and reproductive health-care service needs met, including antenatal and maternal care, information and counselling.

---

**19. Does legislation include any of the following provisions related to violence against women in your country (select all that apply)?**

- ☒ Court injunctions to ensure the safety and security of survivors  
☐ Special prosecutions unit in law enforcement  
☒ Free legal services or assistance  
☐ Financial support to survivors  
☒ Specialized police staff or unit to deal with violence against women  
☒ Services for the person perpetrating violence  
☒ Other (please specify): Access to shelters and other safe housing

---

**20. Does your country have any of the following to protect key populations and people living with HIV from violence (select all that apply)?**

- ☒ General criminal laws prohibiting violence  
☐ Specific legal provisions prohibiting violence against people based on their HIV status or belonging to a key population  
☒ Programmes to address intimate partner violence\*  
☒ Programmes to address workplace violence  
☒ Interventions to address police abuse  
☒ Interventions to address torture and ill treatment in prisons and other closed settings
-

## 5. Community leadership

- Ensure that community-led organizations deliver 30% of testing and treatment services by 2025, with a focus on HIV testing, linkage to treatment, adherence and retention support, and treatment literacy.
- Ensure that community-led organizations deliver 80% of HIV prevention services for populations at high risk of HIV infection by 2025, including for women within those populations.
- Ensure that community-led organizations deliver 60% of programmes to support the achievement of societal enablers by 2025

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**21. Does your country have restrictions for the registration and operation of community-led organizations that affect HIV service delivery (select all that apply)?**

- ☐ Restrictions on registration
- ☐ Territorial restrictions on operations, such as zoning
- ☐ Restrictions on providing services to key populations
- ☐ Cumbersome reporting and other restrictions on operations
- ☐ High cost of registration
- ☐ Other (please specify): \_\_\_\_\_
- ☒ No

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**22. Does your country have laws, policies or regulations that hinder access to funding for HIV-related work by community-led organizations (select all that apply)?**

- ☐ Lack of social contracting or other mechanisms allowing community-led service delivery to be funded from domestic funding
- ☐ Restrictions on accessing funding from "foreign agents" or international donors
- ☐ Regulations governing how organizations can conduct fundraising activities
- ☐ Time-consuming procedures and documentation
- ☐ Eligibility requirements impeding small or recently established organizations to access funding
- ☐ Administrative requirements limiting the capacity of organizations to transfer and implement funds
- ☐ Other (please describe): \_\_\_\_\_
- ☒ No

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### Participation

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**23. Do people living with HIV participate\* in developing national policies, guidelines and/or strategies relating to their health in your country?**

- ☒ Yes
- ☐ No

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**24. Do women living with HIV participate\* in developing national policies, guidelines and strategies relating to prevention of vertical transmission?**

- ☒ Yes
- ☐ No

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**25. Do gay men and other men who have sex with men participate\* in developing national policies, guidelines and/or strategies relating to their health in your country?**

- ☒ Yes
- ☐ No

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**26. Do sex workers participate\* in developing national policies, guidelines and strategies relating to their health in your country?**

- ☒ Yes
- ☐ No

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**27. Do people who inject drugs participate\* in developing national policies, guidelines and strategies relating to their health in your country?**

- ☐ Yes
- ☒ No

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**28. Do transgender people participate\* in developing national policies, guidelines and strategies relating to their health in your country?**

- ☒ Yes
- ☐ No

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**29. Do former or current people in prisons and other closed settings participate\* in developing national policies, guidelines and strategies relating to their health in your country?**

- ☐ Yes
- ☒ No
-

|                                     |                                                                                                                                                      |  |
|-------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------|--|
| 30.                                 | Do young people (aged 15–24 years) participate* in developing national policies, guidelines and strategies relating to their health in your country? |  |
| <input checked="" type="checkbox"/> | Yes                                                                                                                                                  |  |
| <input type="checkbox"/>            | No                                                                                                                                                   |  |

  

|                                                                                                                                                       |                                                                                                                                      |                                            |
|-------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------|
| 30.1                                                                                                                                                  | If yes, do young people participate* in any of the following decision-making spaces in the national HIV response (where they exist)? |                                            |
|                                                                                                                                                       | Does it exist?                                                                                                                       | Do young people participate in this space? |
| Decision-making space                                                                                                                                 |                                                                                                                                      |                                            |
| Technical teams for the development, review and update of national AIDS strategies and plans                                                          | <b>YES</b>                                                                                                                           | <b>YES</b>                                 |
| Technical teams for the development or review of programmes that relate to young people's access to HIV testing, treatment, care and support services | <b>YES</b>                                                                                                                           | <b>YES</b>                                 |
| National AIDS coordinating authority or equivalent, with a broad-based multisector mandate                                                            | <b>NO</b>                                                                                                                            | <b>NO</b>                                  |
| Global Fund to Fight AIDS, Tuberculosis and Malaria (Global Fund) Country Coordinating Mechanism                                                      | <b>YES</b>                                                                                                                           | <b>YES</b>                                 |
| Community advisory body for hospitals, clinics and/or research projects                                                                               | <b>NO</b>                                                                                                                            | <b>NO</b>                                  |
| Other (please specify): _____                                                                                                                         | Yes/No                                                                                                                               | Yes/No                                     |

## 6. Realize human rights and eliminate stigma and discrimination

- Ensure that less than 10% of countries have restrictive legal and policy frameworks that unfairly target people living with, at risk of and affected by HIV. Examples include age of consent laws and laws related to HIV nondisclosure, exposure and transmission, laws that impose HIV-related travel restrictions and mandatory testing, and laws that lead to the denial or limitation of access to services by 2025.
- Invest US\$ 3.1 billion in societal enablers—including protection of human rights, reduction of stigma and discrimination and law reform, where appropriate—in low- and middle-income countries by 2025.
- Ensure that less than 10% of people living with, at risk of and affected by HIV experience stigma and discrimination by 2025

### Law enforcement

31. Have people living with HIV in your country been arrested or prosecuted for nondisclosure, exposure or transmission in the last two years?

☐ Yes

☒ No

32. Have people living with HIV in your country been arrested or prosecuted for vertical exposure or transmission in the last two years?

☐ Yes

☒ No

33. Can possession of a needle or syringe without a prescription be used as evidence of drug use or cause for arrest in your country?

☐ Yes

☒ No

34. Have trans people in your country been arrested or prosecuted for expressions of their gender identity in the past two years?

☐ Yes

☒ No

35. Have sex workers in your country been arrested or prosecuted in relation to selling sex in the past two years?

☐ Yes

☐ No

36. Have people in your country been arrested or prosecuted for consensual same-sex sexual acts in the past two years?

☐ Yes

☒ No

37. Have people in your country been arrested or prosecuted for using or being in possession of any drugs in the past two years?

☒ Yes

☐ No

38. If possession of any drug for personal use is not a criminal offence in your country, are there legal or administrative provisions for drug use and/or possession for personal use that amount to a form of punishment (select all that apply)?

☒ Fines

☐ Loss of identity documents, including passport

☐ Loss of driving license

☒ Quantity thresholds for drug possession do not reflect use

☒ Lack of enforcement of quantity thresholds defined in laws or policies

☐ Lack or limited understanding by law enforcement officials of quantity thresholds or other legal provisions that allow possession and/or use of limited amount of drugs, where these exist

☐ Compulsory treatment in a detention facility

☐ Compulsory treatment in the community

☐ Other (please specify): \_\_\_\_\_

39. Have constitutional or legislative prohibitions of discrimination been successfully used to protect or advocate for transgender people's rights in the last two years in any of the following (select all that apply)?

☐ Courts

☐ Advocacy with government for law or policy reform

☒ Other (please specify): No

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**40. If a change of gender marker is legally recognized, do any of the following barriers limit its application (select all that apply)?**

- ☐ Unclear or vague regulations or application processes
  - ☐ High cost
  - ☐ Gender reassignment surgery a requirement for change
  - ☐ Other (please specify): \_\_\_\_\_
  - ☒ Gender marker change not legally recognized
- 

**41. Have constitutional or legislative prohibitions of discrimination been successfully used to protect or advocate for sex workers' rights in the last two years in any of the following (select all that apply)?**

- ☐ Courts
  - ☒ Advocacy with government for law or policy reform
  - ☐ Other (please specify): \_\_\_\_\_
- 

**42. Have constitutional or legislative prohibitions of discrimination been successfully used to protect or advocate for the rights of people on the basis of their sexual orientation in the last two years in any of the following (select all that apply)?**

- ☒ Courts
  - ☒ Advocacy with government for law or policy reform
  - ☐ Other (please specify): \_\_\_\_\_
- 

**43. Have protective provisions been successfully used to protect or advocate for the rights of people who use drugs in the last two years in any of the following (select all that apply)?**

- ☐ Courts
  - ☐ Advocacy with government for law or policy reform
  - ☒ Other (please specify): No
- 

**44. Do any of the following act as barriers to making use of constitutional or other legislative protections in your country (select all that apply)?**

- ☒ Unclear or lengthy administrative processes
  - ☒ High cost
  - ☒ Difficulties in accessing the judicial system
  - ☒ Lack of legal support
  - ☐ Other (please specify): \_\_\_\_\_
- 

#### Rights Literacy

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**45. In the past two years, have there been training and/or capacity-building programmes for people living with HIV and key populations to raise awareness about their rights in the context of HIV in your country?**

- ☒ Yes, at scale at the national level
- ☐ Yes, at scale at the subnational level (in at least one province/region/district)
- ☐ Yes, one-off activities
- ☐ Yes, at a small scale
- ☐ No

**45.1 If yes:**

**45.1.a Was any public funding provided to support trainings?**

- ☒ Yes
- ☐ No
- ☐ Don't know

**45.1.b Who delivered the trainings (select all that apply)?**

- ☒ Community-led organizations
  - ☒ Government
  - ☐ Other (please specify): \_\_\_\_\_
-

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46. Are there any of the following barriers to providing training and/or capacity-building for people living with HIV and key populations to educate them and raise their awareness about their rights (select all that apply)?

- ☐ Lack of political will
- ☒ Lack of funding
- ☐ Lack of capacity for delivery of trainings
- ☐ Barriers that hinder the target audience in accessing such trainings or capacity-building
- ☐ Others (please describe): \_\_\_\_\_
- 

#### Accountability mechanisms

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47. Are there mechanisms established by the community and/or nongovernmental organizations to record and address individual complaints of HIV-related discrimination (based on perceived HIV status and/or belonging to any key population)? Examples of such mechanisms include traditional cultural structures or nongovernmental organizations trained to address claims through mediation.

- ☒ Yes (please describe): Office of the Public Defender, Jamaica Anti-Discrimination System, Case Management System
- ☐ No
- 

48. Does your country have mechanisms in place for accessing affordable legal services (select all that apply)?

- ☒ Yes, publicly-funded legal aid systems applicable to HIV casework
- ☒ Yes, pro bono legal services provided by private law firms or individual lawyers
- ☒ Yes, legal services provided by (university-based) legal clinics
- ☒ Yes, community paralegals
- ☐ Yes, other (please describe): \_\_\_\_\_
- ☐ No
- 

49. Does your country have any of the following barriers to accessing justice, including through formal and informal mechanisms (select all that apply)?

- ☒ Mechanisms do not function
- ☒ Mechanisms are not sensitive to HIV
- ☒ Affordability constraints for people from marginalized and affected groups
- ☒ Awareness or knowledge of how to use such mechanisms is limited
- ☐ Other (please specify): \_\_\_\_\_
- 

50. Do key populations or people living with or affected by HIV face particular barriers in accessing justice in your country?

- ☐ No
- ☒ Yes—please describe and provide details on the scale (i.e., nationally): Nationally
-

## 7. Universal health coverage and integration

- Invest in robust, resilient, equitable and publicly funded systems for health and social protection systems that provide 90% of people living with, at risk of and affected by HIV with people-centred and context-specific integrated services for: HIV and other communicable diseases; noncommunicable diseases; sexual and reproductive health care; gender-based violence; mental health; palliative care; treatment of alcohol dependence; drug use legal services; and other services they need for their overall health and well-being
- Ensure that by 2025, 45% of people living with, at risk of and affected by HIV and AIDS have access to social protection benefits.
- Ensure that 90% of people in humanitarian settings have access to integrated HIV services.
- Ensure the systematic engagement of HIV responses in pandemic response infrastructure and arrangements, leveraging national HIV strategic plans to guide key elements of pandemic preparedness planning and ensuring that 95% of people living with, at risk of and affected by HIV are protected against pandemics, including COVID-19.

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51. Do any of the following barriers limit access to social protection\* programmes in your country (select all that apply)?

- ☒ Lack of information available on the programmes
  - ☒ Complicated procedures
  - ☒ Fear of stigma and discrimination
  - ☒ Lack of documentation that confers eligibility, such as national identity cards
  - ☒ Laws or policies that present obstacles to access
  - ☒ High out-of-pocket expenses (e.g. transport, time to receive transfers)
  - ☒ Limited access to bank accounts where they are used for delivery of social protection programmes
  - ☐ Limited access to mobile phones where they are used for delivery of social protection programmes
  - ☒ People living with HIV, key populations and/or people affected by HIV are covered by another programme
  - ☐ Other (please specify): \_\_\_\_\_
-